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WHO Country Cooperation Strategy: 2026–2030

BHUTAN



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WHO Country Cooperation Strategy: 2026–2030 – Bhutan

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List of abbreviations and acronyms

AEFI	adverse event following immunization
AMR	antimicrobial resistance
BCG	Bacillus Calmette-Guérin
BHMIS	Bhutan Health Management Information System
BVS	Bhutan Vaccine System
CBOs	community-based organizations
CCS	Country Cooperation Strategy
COPD	chronic obstructive pulmonary disease
CRVS	Civil Registration and Vital Statistics
DHIS-2	District Health Information Software 2
DOTS	directly observed treatment, short-course
ePIS	Electronic Patient Information System
EWARS	Early Warning, Alert and Response System
FCTC	Framework Convention on Tobacco Control
FPOs	Farmer Producer Organizations
FYP	Five-Year Plan
GDP	gross domestic product
GLOFs	glacial lake outburst floods
GNH	gross national happiness
GPW	General Programme of Work
HALE	healthy life expectancy
HiAP	Health in All Policies
HTA	health technology assessment Health in All Policies
IARC	International Agency for Research on Cancer
IEC	information, education and communication
IHR (2005)	International Health Regulations (2005)
JDWNRH	Jigme Dorji Wangchuck National Referral Hospital
LDCs	least developed countries
MBBS	Bachelor of Medicine and Bachelor of Surgery
MMR	maternal mortality ratio
MR	measles-rubella
MDR-TB	multidrug-resistant tuberculosis
NAP	National Adaptation Plan

NCD	noncommunicable disease
NHS	National Health Survey
NIS	National Immunization Strategy
OPV	oral polio vaccine
PEN	package of essential NCD interventions
PHC	primary health care
PLHIV	people living with HIV
RMNCAH	reproductive, maternal, newborn, child and adolescent health
SCCI	Service with Care and Compassion Initiative
SDG	Sustainable Development Goal
SEARHEF	South-East Asia Regional Health Emergency Fund
STAR	Strategic Toolkit for Assessing Risks
TAPS	tobacco advertising, promotion and sponsorship
UHC	universal health coverage
UNDP	United Nations Development Programme
UNESCO	United Nations Educational, Scientific and Cultural Organization
UNFPA	United Nations Population Fund
UNICEF	United Nations Children's Fund
UNSDCF	United Nations Sustainable Development Cooperation Framework
VDCP	Vector-Borne Disease Control Programme
VIA	visual inspection with acetic acid
WASH	water, sanitation and hygiene
WFP	World Food Programme
WHO	World Health Organization
WSP	water safety planning

Foreword by H.E. Lyonpo Tandin Wangchuk

The World Health Organization has since long been a key partner in strengthening Bhutan's health sector. It is with great pride and optimism that I share this message on the launch of the WHO Country Cooperation Strategy (CCS) 2026–2030 for Bhutan.

The period from 2020 to 2025 was one of extraordinary trials and transformations for Bhutan's health sector. Like much of the world, we confronted the profound challenges of the COVID-19 pandemic, which tested the resilience of our health system and demanded a whole-of-society response. Bhutan's swift and compassionate management of the pandemic, including one of the fastest vaccination campaigns globally, demonstrated not only the strength of our public institutions but also the unwavering solidarity of our people.



Even as we responded to the pandemic, we continued to advance long-standing national health priorities. We achieved remarkable progress in reducing maternal and child mortality, sustaining high immunization coverage, and integrating mental health and traditional medicine into mainstream health care. Notably, Bhutan became the first country in the South-East Asia Region to meet the 2030 interim targets for cervical cancer elimination – an achievement that testifies to our vision and our values.

As we look to the future, our focus must now shift from recovery to resilience. The 13th Five-Year Plan (2024–2029) outlines our vision to build a more equitable, inclusive and sustainable health system. This health system must be better prepared for future emergencies and better able to meet the needs of our ageing population. It must also address the rising burden of noncommunicable diseases, and endeavour to close persistent gaps in access and quality of care, especially in rural and remote communities.

Guided by the unique principles of Gross National Happiness observed in Bhutan, our path forward emphasizes not only physical but also mental, social and spiritual well-being. Digital health innovations, workforce reforms, and sustainable financing mechanisms will be at the heart of our strategy moving forward. Equally important is our unwavering commitment to equity, inclusion, and the principle of leaving no one behind.

In this endeavour, the World Health Organization remains an indispensable partner. Over four decades of collaboration have helped shape Bhutan's health journey, from immunization campaigns and disease elimination to health system strengthening and pandemic preparedness. The CCS 2026–2030 reflects not only WHO's technical leadership but also its deep understanding of Bhutan's values, priorities and aspirations.

I commend the WHO Country Office in Bhutan for facilitating an inclusive and consultative process in developing this Strategy. On behalf of the Royal Government of Bhutan, I reaffirm our commitment to work closely with WHO and all partners in the spirit of solidarity, shared responsibility, and common purpose. Together, let us advance our collective goal of achieving health and happiness for all Bhutanese people.

A handwritten signature in black ink, belonging to H.E. Lyonpo Tandin Wangchuk. The signature is stylized and cursive, written on a white background.

H.E. Lyonpo Tandin Wangchuk
Minister of Health
Royal Government of Bhutan
Thimphu

Foreword by the Officer-in-Charge for South-East Asia



The Kingdom of Bhutan's pursuit of health equity, resilience, and universal health coverage is anchored in the philosophy of Gross National Happiness and offers a compelling model for the South-East Asia Region and beyond. The country's achievements in health are borne out of visionary leadership, strong community engagement, and an abiding commitment to inclusive and people-centred development.

The WHO Country Cooperation Strategy (CCS) 2026–2030 builds on this foundation and provides a strategic framework to guide our partnership over the next five years. Informed by an independent evaluation, and shaped through consultation within the country and across the three levels of WHO, the CCS aligns with Bhutan's 13th Five-Year Plan, the UN Sustainable Development Cooperation Framework, and WHO's Fourteenth General Programme of Work. The CCS is a manifestation of our shared ambition to sustain health gains while adapting to new challenges in an increasingly complex global environment.

Bhutan's accomplishments range from its exemplary COVID-19 response and pioneering digital health innovations to its global leadership in eliminating cervical cancer, and highlights what is possible through country-led and values-driven approaches. These successes are both national achievements and regional accelerators, advancing our collective progress toward the Sustainable Development Goals.

The CCS focuses on four strategic priorities: expanding multisectoral action on noncommunicable diseases and mental health; strengthening communicable disease control; advancing equitable universal health coverage; and enhancing health emergency preparedness and response. These priorities align with the WHO South-East Asia Regional Roadmap for Results and Resilience (2024–2029) and will be key to building health systems that are future-ready, inclusive, and impact-oriented.

WHO remains a committed partner in Bhutan's health journey. Through this CCS, we reaffirm our support to the Royal Government of Bhutan in delivering transformative results through technical excellence, capacity building, and strategic collaboration.

Together, we will continue to drive progress toward a healthier, more equitable, and resilient future for all.

A handwritten signature in black ink, appearing to be 'C. Boehme'.

Dr Catharina Boehme
Officer-in-Charge
WHO South-East Asia

Message from the WHO Representative

Bhutan's pursuit of health equity and universal health coverage continues to inspire not only the South-East Asia Region but also the global health community. The WHO Country Cooperation Strategy (CCS) 2026–2030 marks an important new chapter in our long-standing partnership with the Royal Government of Bhutan. It reaffirms our shared vision of building a health system that is inclusive, adaptive and resilient, and capable of addressing persistent challenges while anticipating emerging health threats.

The CCS 2026–2030 builds on the remarkable achievements of the previous strategy cycle. Bhutan's extraordinary COVID-19 response, global leadership in cervical cancer elimination, and unwavering commitment to universal health coverage are emblematic of a health system guided by compassion, innovation and national values rooted in the philosophy of Gross National Happiness.

This new strategy is the product of an inclusive and evidence-informed process, grounded in the findings of the independent evaluation of CCS 2020–2025, and developed in close consultation with the Ministry of Health, development partners, civil society and other stakeholders. While the previous CCS focused on integrated primary health care, maternal and child health, and strengthening systems to respond to communicable diseases, the CCS 2026–2030 sharpens its focus on four Strategic Priorities:

- (1) Expanding multisectoral action on noncommunicable diseases and mental health;
- (2) Strengthening communicable disease control and prevention;
- (3) Advancing equitable universal health coverage; and
- (4) Enhancing health emergency preparedness and response.

These Strategic Priorities are fully aligned with WHO's Fourteenth General Programme of Work (GPW14), Bhutan's 13th Five-Year Plan, and the UN Sustainable Development Cooperation Framework (UNSDCF) 2024–2028. They are designed to translate global commitments into meaningful national action that supports Bhutan's aspirations to build a healthier, more equitable and more resilient health system.

The CCS also reflects Bhutan's pioneering efforts in digital health transformation, including innovations such as the electronic Patient Information System, and it recognizes the urgent need to address workforce and financing gaps to sustain these advances.

The WHO Country Office in Bhutan remains fully committed to supporting the Royal Government in delivering quality, inclusive and people-centred health-care services. It is both an honour and a privilege to walk this path together, with the government, our development partners and, most importantly, the people of Bhutan.

We look forward to strengthening our partnership in this CCS period and beyond, working together toward the shared goal of health and well-being for all.



Dr Bhupinder Kaur Aulakh
WHO Representative to Bhutan
World Health Organization

Executive summary

The Country Cooperation Strategy (CCS) 2026–2030 reinforces the strategic agenda of WHO for Bhutan, aiming to enhance the effectiveness, efficiency and quality of its collaboration with the Royal Government of Bhutan. Building on a partnership that began in 1982, this CCS seeks to ensure the greatest possible contribution to the health and development outcomes of Bhutan. It aligns with the country's 13th Five-Year Plan (2024–2029), the Sustainable Development Goals (SDGs), the United Nations Sustainable Development Cooperation Framework (UNSDCF) 2024–2028 and the WHO Fourteenth General Programme of Work (GPW 14, 2025–2028).

Bhutan has made remarkable progress in health outcomes. Maternal mortality dropped significantly from 255 per 100 000 live births in 2000 to 53 in 2023 while under-five mortality declined from 84 per 1000 live births in 2000 to 19.5 in 2023. Immunization coverage remains robust at over 99% and institutional delivery rates have reached 98.5%, as per National Health Survey (NHS) 2023. Almost all households (99.7%) and the overall population (99.7%) have access to improved drinking water sources.

Bhutan is also a regional leader in cervical cancer elimination. It has successfully integrated human papillomavirus (HPV) vaccination for both boys and girls into its National Immunization Programme. Bhutan became the first country in the WHO South-East (SE) Asia Region to meet the 2030 interim targets for cervical cancer elimination. As of 2023, HPV vaccination coverage exceeded 90% while HPV screening and treatment coverage reached 90.81% and 91.98% respectively (Flagship Report, 2023).

However, challenges persist. The sustainability of the health system is a significant concern amid escalating health-care costs, advancing health technologies, increasing public expectations, a rapid rise in noncommunicable diseases (NCDs) and an ageing population. NCDs account for 70% of the total number of deaths in Bhutan. The country saw an increase in tobacco use, reaching 31.4% in 2023, alongside a hypertension rate of 30.1% in the same year, and rise in cancer cases. Communicable diseases such as tuberculosis remain a concern, with an incidence rate of 126 cases per 100 000.

Workforce shortages – including a 68% gap for specialists, 42% for general physicians and nursing staff – combined with geographical inequities in health care access continue to strain the health system of Bhutan. These shortages are compounded by challenges in ensuring the quality and competency of health personnel, particularly at the primary care level and in remote areas. Emerging climate risks, such as glacial lake outburst floods (GLOFs) and re-emerging vector-borne diseases, further strain the health system. While Bhutan ensures wide health coverage through an extensive distribution of health care delivery systems, the accessibility of these health facilities for people with disability needs to be strengthened.

In Bhutan, the prevalence of disability (excluding children under the age of five) is estimated at 6.8%. The most common functional limitations include difficulties with self-care, followed by impairments in hearing and mobility. Aligned with global trends, the prevalence of disability is higher among those aged 65 years and older (27.4%) and those with low income (11.2% in the lowest income group).¹ While information on NCDs among persons with disability in Bhutan is limited,

¹ Ministry of Health, Royal Government of Bhutan (2024). 5th National Health Survey: Integrated Stepwise Household Survey 2023.

global studies demonstrate a higher incidence of noncommunicable diseases among persons with disability, compared with persons without disability. For example, persons with disability face more than double the risk of developing conditions such as diabetes, stroke or depression. Furthermore, there is evidence that all major NCD risk factors – such as tobacco use, physical inactivity, harmful use of alcohol and unhealthy diets – disproportionately affect persons with disability.²

The dual burden of communicable and noncommunicable diseases, alongside Bhutan’s unique geographical and demographic context, necessitates targeted strategies to ensure equitable health care. The rising older population underscores the need for lifelong care, expanded maternal and newborn services, robust immunization programmes, and enhanced child and adolescent health initiatives. Investing in emergency preparedness and response systems, digital innovations and multisectoral collaboration will be critical to sustaining progress and adapting to emerging challenges, ensuring a healthier and more inclusive future for Bhutan. Globally, one in three people is estimated to be living with a health condition that would benefit from rehabilitation. Rehabilitation services need to be an integral part of the treatment at the primary health care (PHC) level.

There are significant shortages in numbers of medical and health professionals, particularly specialists, along with insufficient capacity development and training opportunities for staff, as well as severe understaffing, exacerbated by increasing workload burdens due to international migration of staff.

Four Strategic Priorities have been identified following a series of consultations with Bhutan’s Ministry of Health and other relevant stakeholders. They are based on a critical analysis of the country’s needs and WHO’s comparative advantage in addressing those needs.

The CCS 2026–2030 will be operationalized through biennial work plans and robust monitoring frameworks, leveraging strong partnerships.

Strategic Priorities for CCS 2026–2030

STRATEGIC PRIORITY 1: Expand multi-sectoral collaboration to address NCDs and Mental Health across life-course	STRATEGIC PRIORITY 2: Prevention and Control of Communicable Disease	STRATEGIC PRIORITY 3: Achieve Equitable Universal Health Coverage (UHC)	STRATEGIC PRIORITY 4: Strengthen Emergency Preparedness and Response
1. NCD and risk factors management and control 2. Cancer 3. Mental health	1. Malaria Prevention & Control including other NTDs 2. HIV/STI 3. Tuberculosis 4. Infectious Disease Prevention & Control	1. Comprehensive Primary Health Care & Immunization 2. Digital Health and Telemedicine 3. Human Resources for Health 4. Sustainable Health Financing	1. Health Emergency Prevention & Preparedness 2. Zoonotic and emerging diseases prevention and preparedness

² World Health Organization (2022) Global Report on Health Equity for Persons with Disabilities.

1. Introduction

This document is the WHO Country Cooperating Strategy 2026–2030 for Bhutan. It serves as the strategic framework guiding WHO’s partnership with the Royal Government of Bhutan, particularly with the Ministry of Health (MoH), to strengthen the country’s health system and advance national health priorities. Rooted in the WHO Fourteenth General Programme of Work (GPW 14) and aligned with the 13th Five-Year Plan (FYP 2024–2029) of Bhutan, the CCS is designed to support the country in achieving universal health coverage (UHC), enhancing public health preparedness and addressing emerging health challenges.

Building upon the achievements and lessons learnt from the CCS 2020–2025, this Strategy reflects a consultative, evidence-based approach, incorporating insights from the independent evaluation of the previous CCS cycle, national policy frameworks and key health stakeholders. The development process included consultations with the Ministry of Health, the three levels of WHO (Country Office, Regional Office and headquarters), UN agencies, civil society organizations (CSOs) and development partners to ensure that the CCS responds effectively to the evolving health needs of Bhutan.

As part of WHO’s differentiated approach to country engagement, Bhutan has been designated as a Typology C country, indicating the need for moderate technical support to address national health priorities. This classification ensures that WHO provides targeted policy, strategic and technical assistance that aligns with the capacity, resources and evolving health sector needs of Bhutan.

This document is intended for policy-makers, government institutions, development partners, WHO teams at the country, regional and headquarters levels, civil society organizations, and other stakeholders involved in the health sector of Bhutan. The CCS 2026–2030 will serve as the foundation for WHO’s engagement in Bhutan, providing strategic direction for technical collaboration, capacity-building and policy support. The document outlines key Strategic Priorities, implementation mechanisms and monitoring frameworks to enhance Bhutan’s health system resilience and improve health outcomes for all Bhutanese people.

Grounded in the principles of equity, universality and inclusiveness, the CCS is designed to:			
strengthen health systems to ensure universal health coverage (UHC) and equitable access to quality services	address health inequities by targeting the needs of vulnerable and marginalized populations	strengthen resilience to health emergencies preparedness and response, particularly those influenced by climate change and other emerging threats	foster multisectoral collaboration to address the broader social, economic and environmental determinants of health

2. Country context and health situation

Bhutan at a glance

Category	Details
Geography	Landlocked in the eastern Himalayas, bordered by India in the south and China in the north; diplomatic neutrality and peaceful coexistence
Population	~750 000; over 40% of the country population is under 24 years; the elderly population to rise from 6% (2023) to 17.3% (2050); fertility rate: 2
Economy	Hydropower exports dominate gross domestic product (GDP); GDP growth: -10.22% (2020), 5.21% (2022), 4.88% (2024); graduated from the least developed country (LDC) status in 2023
Social indicators	Literacy rate: 72% (2022); life expectancy: 74.9 years (2021); primary enrolment: 93.2%; rural electrification: nearly 100%
Health system	Free health-care services at points of care; challenges include health workforce and infrastructure shortages, access barriers and climate-related risks.
Health status	Maternal mortality ratio (MMR): 53/100 000 LB; infant mortality rate (IMR): 15.2/1000 LB; immunization coverage: over 99%; near elimination of malaria, rubella, kala-azar, rabies and leprosy; NCDs (~70% of total deaths)

2.1 Geopolitical and economic situation

Bhutan, a small kingdom nestled in the eastern Himalayas, holds a unique geopolitical position in South Asia. Bordered by two global powers, India and China, Bhutan has historically prioritized diplomatic neutrality and peaceful coexistence. Bhutan's reliance on hydropower exports to India underscores the importance of this partnership, with hydropower accounting for a major share of its GDP. Renowned for its gross national happiness (GNH) development philosophy, Bhutan's approach aligns closely with the Sustainable Development Goals (SDGs), emphasizing holistic well-being, alongside economic growth.

As a lower-middle-income country with a GDP per capita of US\$ 3718, Bhutan achieved remarkable progress in poverty reduction. ***Extreme poverty, defined as living on less than US\$ 2.15/day, was nearly eradicated, and the proportion of people living below US\$ 3.65/day (lower-middle-income country poverty line) and US\$ 6.85/day (upper-middle-income country poverty line)³ decreased significantly, reflecting improved socioeconomic conditions.*** However, challenges persist. These include inequalities, multihazard vulnerabilities and regional disparities; the pandemic also exacerbated labour market issues and outmigration trends.⁴

Bhutan's demographic structure presents both opportunities and challenges: over 40% of its population is under 24 years while the elderly population is expected to rise from 6% in 2023 to 17.3% by 2050.⁵ Concurrently, rural-to-urban migration continues to increase, straining urban social services, housing affordability and employment opportunities.

3 Country Profiles

4 World Bank Document

5 https://bhutan.un.org/sites/default/files/2024-09/Bhutan_UNSDCF%202024-2028.pdf

While Bhutan has experienced impressive economic growth, averaging 7.5% annually, since the 1980s, the real GDP growth has slowed to around 3.5% over the past decade due to structural constraints and post-pandemic recovery challenges.⁶ Nonetheless, this growth trajectory has contributed to notable socioeconomic development and poverty reduction. ***In December 2023, Bhutan graduated from the least developed countries (LDCs) category, a milestone marking its transition to greater economic stability.***⁷

The following section delves into the health situation of the country, analysing the key achievements and areas requiring attention to support its continued journey towards universal health coverage and sustainable well-being.

2.2 Key equity issues

The rugged terrain limits health care access, increases transportation cost and causes delays in seeking care. These challenges result in poorer health outcomes, higher rates of preventable diseases, and restricted maternal and nutritional health services, especially in rural and high-altitude areas, for people living in vulnerable conditions or circumstances. Women in these regions face heightened risks during pregnancy and childbirth, indicating the need for equitable health care delivery. Although over 90% of the population lives within 30 minutes of a health-care facility (NHS 2023), service quality and availability vary widely. Urban centres such as Thimphu and Paro benefit from better infrastructure and specialist services while a large majority of people from rural districts have to bypass the nearest health facility due to limited services.

Overall, 91.2% of people surveyed as part of the National Health Survey 2023 (NHS 2023) reported satisfaction with the health services at the facilities, with satisfaction levels higher in rural (93.6%) than in urban (87.7%) settings. For instance, ***institutional delivery rates are near universal in urban areas (99.6%) but remain lower in districts such as Zhemgang (86.9%), Tsirang (91.9%) and Trongsa (94%), despite an overall 98% increase nationally (NHS 2023).*** However, certain groups, including the elderly and people with disability, still face barriers to accessing health care due to physical, social and systemic challenges. Geographical isolation, mobility constraints, inadequate specialized services and societal perceptions further limit equitable access.

A 2017 survey found that only 16% of elderly respondents reported good health while 44% rated their health as poor, with mobility limitations and a lack of geriatric care restricting timely medical attention.⁸ Similarly, negative attitudes and misconceptions among health-care providers contribute to the exclusion of persons with disability from mainstream services.⁹ Further, increased emigration of people and disintegration of the traditional family structure, along with lack of family support, pose threat to the ageing population.

Socioeconomic factors such as income, education and geographical disparities significantly influence health equity in Bhutan, exacerbating challenges for marginalized groups, including people living in remote and high-altitude regions, persons with disability, the elderly, people living in poverty and key populations such as those living with HIV or affected by substance use. Income disparities

6 World Bank. 2023. Bhutan development update: Fiscal sustainability for inclusive growth. April 2023.

7 <https://www.un.org/development/desa/dpad/2023/bhutan-graduates-from-ldc-status/>

8 https://www.nsb.gov.bt/wp-content/uploads/dlm_uploads/2022/02/Understanding-the-situation-of-elderly-citizens-in-Bhutan.pdf

9 https://www.nsb.gov.bt/wp-content/uploads/dlm_uploads/2022/02/Understanding-the-situation-of-elderly-citizens-in-Bhutan.pdf

remain stark, with poverty rates ranging from 1.5% in urban Thimphu to 41.4% in rural Zhemgang,¹⁰ limiting access to health care for rural populations. Similarly, educational attainment directly impacts health outcomes, as reflected in Bhutan's adult literacy rate of 72%, with a notable gender gap (79% for men vs 64% for women).¹¹ Other marginalized groups also face unique health challenges. Drug users face stigma and limited treatment options while tuberculosis patients struggle with treatment adherence and associated economic burdens. Even though the HIV prevalence remains low, as compared with global averages, people living with HIV (PLHIV) face stigma, delayed diagnoses and limited access to timely treatment.

Key cross-cutting issues include geographical and economic barriers, limited integration of health-care services, social stigma and resource constraints, which collectively hinder equitable health outcomes. Adolescents and youths face a range of challenges, including tobacco use, as well as electronic cigarettes and vaping, rising rates of mental health issues such as depression and anxiety, significant gaps in sexual and reproductive health knowledge and service access, concerning patterns of substance abuse, and inadequate levels of nutrition and physical activity. It is estimated that **18.3% of Bhutanese adults, including 22.3% of men and 14.7% of women, are not meeting WHO physical activity recommendations, and 74.6% of the population are consuming fewer fruits and vegetables than recommended dietary guidelines.**

Although Bhutan has made efforts to promote physical activity, including major initiatives launched between 2016 and 2018, data on long-term improvements remain limited. The 2014 Bhutan STEPS Survey reported a reduction in sedentary behaviour, with the mean sitting time per day decreasing from 174.2 minutes¹² in 2007 to 148 minutes in 2014.¹³ The average daily intake of fruits and vegetables consumed per day is only 1.6 servings, highlighting insufficient nutritional intake. Additionally, 5.1% of children under five experience wasting, indicating acute malnutrition, while 17.9% are stunted, reflecting chronic undernutrition (NHS 2023). Emerging issues such as digital health risks are also gaining prominence, particularly among urban youths. This refers to health challenges arising from excessive screen time, internet addiction, cyberbullying and exposure to harmful online content.¹⁴

2.3 SDG alignment

Bhutan has demonstrated significant progress in aligning its health priorities with SDGs, particularly SDG 3 that aims to ensure good health and well-being for all. As of 2023, Bhutan is advancing on approximately 58% of the measurable SDG indicators (72 indicators) for which sufficient data are available, reflecting its commitment to sustainable development (UNSDPF).¹⁵

10 PAR-2022-for-WEB.pdf

11 <https://data.worldbank.org/indicator/SE.ADT.LITR.ZS?locations=BT>

12 Cowan M, Dorji G, Pelzom D. Report on 2007 STEPS Survey for risk factors and prevalence of noncommunicable diseases in Thimphu. Thimphu (Bhutan): Ministry of Health; 2009.

13 <https://pmc.ncbi.nlm.nih.gov/articles/PMC8321908/#b19-kjfm-20-0059>

14 Why Bhutan should regulate social media | Kuensel Online

15 UNITED NATIONS SUSTAINABLE DEVELOPMENT COOPERATION FRAMEWORK (UNSDCF) for BHUTAN 2024–2028

Bhutan has made notable advancements in health outcomes.	Maternal and child health: Bhutan's maternal mortality ratio dropped from 255 per 100 000 live births in 2000 to 53 per 100 000 in 2023, aligning with SDG Target 3.1. Similarly, under-five mortality declined from 84 per 1000 live births in 2000 to 19.5 per 1000 in 2023, supported by improvements in skilled birth attendance and institutional deliveries. Neonatal mortality has also declined significantly since 2000, to 6.9 per 1000 live births in 2023. Policies such as the Policy to Accelerate Mother and Child Health Outcomes (developed in 2020) have further strengthened Bhutan's ability to achieve maternal and child health targets under SDG Targets 3.1 and 3.2.
Communicable diseases: Bhutan has achieved substantial progress in containing diseases such as measles, rubella, HIV/AIDS, tuberculosis, malaria, antimicrobial resistance (AMR), hepatitis B and neglected tropical diseases (NTDs) such as rabies, leprosy and kala-azar and AMR, aligning with SDG Target 3.3. Enhanced surveillance, immunization programmes and targeted interventions have been pivotal in achieving this progress.	Noncommunicable diseases (NCDs): Bhutan is tackling its growing NCD burden, which accounts for over 70% of its total deaths. Public health initiatives targeting tobacco use, physical inactivity and unhealthy diets are aligned with SDG Target 3.4, which seeks to reduce premature mortality from NCDs by one third by 2030.
Universal health coverage (UHC): Bhutan's three-tier health system provides free and equitable health-care services, aligning with SDG Target 3.8. The UHC Service Coverage Index (SCI) improved from 53 in 2015 to 60 in 2021.¹⁶ However, total health expenditure as a percentage of GDP remains around 4.5%, highlighting the need for increased investments to fully achieve UHC.¹⁷	Health workforce: Bhutan increased the density of medical doctors, nurses and midwives from 16.7 per 10 000 in 2014 to 24.8 per 10 000 in 2024.¹⁸ These achievements align with SDG Target 3.c, which emphasizes strengthening the health workforce in developing countries.
Disability inclusion and equity: Bhutan's recent ratification of the United Nations Convention on the Rights of Persons with Disabilities (UNCPRD) strengthens its commitment to inclusive health. This reinforces the country's aim to ensure universal accessibility across all health facilities and support equitable access to care for persons with disability – crucial to achieving the SDG principle of “leaving no one behind”.	Mental health: Bhutan has prioritized mental health, addressing rising rates of behavioural disorders. Investments in mental health services align with SDG Target 3.4, underscoring the government's commitment to holistic well-being, including the recent PEMA Symposium 2024 with the theme “Towards building people-centred mental health care through resilient and supportive society”, organized in collaboration with the PEMA Secretariat and the World Health Organization under the leadership of Her Majesty The Gyaltsuen Jetsun Pema Wangchuck.
Sanitation and Hygiene: Bhutan's declaration of being open defecation-free in 2022 demonstrates progress under SDG Target 6.2, ensuring access to adequate sanitation and hygiene.	

16 <https://iris.who.int/bitstream/handle/10665/374059/9789240080379-eng.pdf?sequence=1>

17 National Health Accounts, 2021 <https://moh.gov.bt/wp-content/uploads/2025/01/NHA-Report-2021.pdf>

18 9789290229636-eng .pdf

2.4 Health situation in the country

This section explores Bhutan's health context, highlighting key indicators, disease trends, health system dynamics, and alignment with national and global priorities.

2.4.1 Key health indicators

The table below provides a comprehensive overview of Bhutan's status on key health indicators, comparing them with global and regional averages and relevant SDG 2030 targets. Details on specific health indicators and trends are provided in Appendices Section 8.1.

Health indicators	Global average	WHO regional average	SDG 2030 Target	Bhutan's status (2023) ¹⁹
General fertility rate <i>Number of births per 1000 women aged 15–49 years</i>	–	–	–	75.2
Total fertility rate (TFR)* <i>Average number of children per woman</i>	2.3 (2022)	2.2 (2022)	–	2
Adolescent birth/fertility rate (A/BFR)** <i>Number of births per 1000 adolescent women aged 15–19 years</i>	41.3 (2023)	24.3 (2023)	–	18.6
Neonatal mortality rate (NMR)** <i>Number of deaths before reaching the age of one month per 1000 live births</i>	17.55 (2021)	17.28 (2021)	12	6.9
Infant mortality rate (IMR)** <i>Number of U1 deaths per 1000 live births</i>	28.42 (2021)	23.87 (2021)	–	15.2
Under-five mortality rate (U5MR)** <i>Number of U5 deaths per 1000 live births</i>	38.09 (2021)	28.64 (2021)	25	19.5
Maternal mortality ratio (MMR)** <i>Number of maternal deaths per 100 000 live births</i>	223 (2020)	117 (2020)	70	53
Skilled birth attendance (SBA)*** <i>Percentage of births attended by a skilled health worker</i>	86 (2023)	86 (2023)	–	98.5
Institutional delivery (ID)*** <i>Percentage of deliveries at health facilities</i>	79 (2023)	82 (2023)	–	98
Exclusive breastfeeding (0–6 months) (EB)*** <i>Percentage of infants exclusively breastfed for the first six months</i>	48 (2023)	60 (2023)	–	69.4
Child vaccination coverage <i>Percentage of children aged 12–23 months fully vaccinated</i>	–	–	–	99.4

Source for global and WHO regional average:

*World Bank Database

**WHO Global Health Observatory Data Repository

***UNICEF Global Database

****World Obesity Atlas 2024

19 The National Health Survey (NHS) 2023 is a comprehensive survey conducted by Bhutan's Ministry of Health to assess the health status, trends and challenges of the population. The survey provides critical data on key health indicators, including maternal and child health, noncommunicable diseases, health care access, and risk factors such as tobacco use and physical inactivity. It serves as a foundational resource for policy-making, programme evaluation and strategic

2.4.2 Health challenges in Bhutan

• Disease burden and trends

- NCDs account for over **70% of all deaths in Bhutan**. The remaining deaths are due to communicable, maternal, perinatal and nutritional conditions (18%), and injuries (10%).²⁰
- Risk factors such as tobacco use, physical inactivity and unhealthy diets are on the rise; tobacco use increased from 23.9% in 2019 to 31.4 % in 2023, partly attributed to the relaxation of tobacco control measures during the COVID-19 pandemic.
- Prevalence of high blood pressure has increased to 30.1% in 2023 from 28% in 2019.
- Alcohol consumption remains another significant concern, with 40.2% of respondents identified as current drinkers.
- Over 33% of adults (rural: 33.5% and urban: 33.1%) are overweight and 12.5% (rural: 13.7% and urban: 11.7%) categorized as obese. Trashiyangtse district reported the highest overweight prevalence rate at 39.7%, followed by Trongsa (39%), in comparison to other districts.
- Cancers constitute a significant proportion of Bhutan's NCD burden. Bhutan reported 638 new cancer cases (male: 356; female: 282) in 2022,²¹ with stomach, lung, cervix uteri, oesophagus and liver as the top five cancers.

These trends underscore the need for an integrated approach to managing NCDs and addressing risk factors to improve the quality of life, especially in later years.

• Maternal and child health

- According to the National Health Survey 2023, anaemia affects 33.3% of pregnant women, posing risks to maternal and neonatal health, including low birth weight, preterm delivery and maternal complications.
- Disparities between urban and rural areas persist with regard to accessing maternal and child health services.
- Despite impressive reduction in MMR, Bhutan is not on track to meet the SDG 2030 target of two thirds reduction in MMR from the 2010 baseline, as per the United Nations Maternal Mortality Estimation Inter-Agency Group (UN MME).

• Adolescent health

- Among adolescents (aged 10–19), anaemia prevalence remains high at 36.5%, highlighting broader nutritional challenges and the need for targeted interventions.
- Additionally, vitamin D deficiency among adolescents was reported to be as high as 93.9%, as per NHS 2023, with higher prevalence among girls (96.3%) than among boys (92.5%).

planning in Bhutan's health sector, guiding efforts towards achieving universal health coverage and aligning with the Sustainable Development Goals.

20 <https://ncdportal.org/CountryProfile/GHE110/BTN>

21 <https://gco.iarc.who.int/media/globocan/factsheets/populations/64-bhutan-fact-sheet.pdf>

- **Geographical and infrastructure access to health care**

- While most of the rural population lives within 30 minutes from the nearest health facility, over 21% of them live more than an hour away from the nearest health facility.
- Three referral hospitals (one national and two regional), 51 hospitals and 187 primary health centres largely serve the entire country. However, these are not evenly distributed across the country.
- As of 2021, Bhutan had approximately 2.2 hospital beds per 1000 people.²²
- Total health expenditure stands at 4.5% of GDP (National Health Accounts 2021).
- Essential medicine stockouts are reported at 40% (Essential Medicines Annual Report 2023).
- Households allocate 15.4% of their expenses to out-of-pocket spending (National Health Accounts 2021).
- Bhutan has initiated efforts to enhance accessibility at health facilities for persons with disability, in line with its commitment under the United Nations Convention on the Rights of Persons with Disabilities (UNCPRD).

- **Mental health issues**

- According to NHS 2023, 1.6% of the population attempted suicide.
- As revealed by Bhutan administrative data, anxiety and depression account for over 55% of the reported cases.
- As of 2021, mental health disorders affected 156 per 10 000 population.²³

Refer to Section 8.1 in Appendices for data on the number of mental health cases.

2.4.3 Human resource for health in Bhutan

- The country is facing a shortage of human resources for health-care services, especially at the specialist levels, with 68% and 42% gaps in the required number of specialists and general physicians respectively (Health Flagship Blueprint, 2020–2023). This challenge has been further intensified by the increased migration of health workers overseas following the COVID-19 pandemic.
- As of 2024, Bhutan had a combined density of 24.8 doctors, nurses and midwives per 10 000 population, reflecting notable progress from 16.7 per 10 000 in 2015 (SE Asia Region UHC Monitoring Report 2024).
- The majority of health-care workers were concentrated in major urban areas such as Thimphu and Paro.
- Speciality care gaps reveal critical shortages in specialities such as psychiatry, oncology and paediatric surgery, with many districts lacking permanent specialist doctors. This leads to heavy reliance on visiting specialists from regional hospitals.
- Other key challenges involve high turnover rates in remote areas, with some reaching 30%–40% annually; limited postgraduate training capacity within the country; ageing workforce

22 https://www.ceicdata.com/en/bhutan/social-health-statistics/bt-hospital-beds-per-1000-people?utm_source=chatgpt.com

23 https://moh.gov.bt/wp-content/uploads/2025/01/Annual-Health-Bulleti-2022_Link-3.pdf

in some specialities; and difficulty in attracting specialists to permanent positions outside the capital. In addition to workforce shortages and maldistribution, there are gaps in the quality and competency of health personnel. These are linked to limitations in pre-service and in-service education as well as the need for strengthened regulatory frameworks.

- This further impacts the health-care infrastructure, as some primary health centres operate below capacity due to staffing shortages.

2.4.4 Health systems and policies

Bhutan's health system operates on a three-tier model designed to ensure equitable access to health care for all citizens. At the foundational level are the primary health centres and outreach clinics, which deliver primary health care services, especially in rural and remote areas. These units are complemented by district hospitals, which provide secondary care and act as referral centres for more complex cases. At the pinnacle of the system stands the Jigme Dorji Wangchuck National Referral Hospital (JDWNRH) in Thimphu, which handles tertiary care and advanced medical needs. This tiered structure reflects Bhutan's commitment to universal health coverage (UHC) and equitable health care access. The government refers complicated cases requiring special attention beyond the ambit of the National Referral Hospital to facilities in India.

Bhutan's commitment to UHC is enshrined in its Constitution, which guarantees free health-care services for all citizens as a fundamental right. The country's health financing policy²⁴ relies predominantly on government funding, supplemented by international aid and development assistance, to sustain the provision of free health-care services at the point of care.

Bhutan has made significant strides in strengthening its health system. For instance, the Service with Care and Compassion Initiative (SCCI) has improved patient-centred care delivery, particularly with regard to the management of NCDs. Bhutan has also implemented comprehensive screening protocols for gastric, cervical and breast cancers, along with policies such as the Cancer Control Strategy (2019–2025) and Guidelines for Screening (2020–2023), which focus on evidence-based, people-centred care.²⁵ Additionally, the creation of the National Medical Service²⁶ in 2023 represents a major reform aimed at streamlining health care delivery, reducing dependency on overseas referrals and enhancing domestic capacity for specialized care. The Service is designed to improve efficiency, equity and quality across all levels of the health system, reflecting Bhutan's long-term vision for health system sustainability.

The Bhutan Health Management Information System (BHMIS) compiles routine health, morbidity and mortality data into the District Health Information Software 2 (DHIS-2) platform. Health facilities input monthly activity, morbidity and mortality reports as well as annual household data into this web-based system. A separate information system is in place for notifiable diseases and potential outbreaks, including the National Early Warning, Alert and Response System (NEWARS), which combines indicator-based and event-based surveillance. At the national level, aggregate data are used to track indicators for monitoring the progress of various programmes.

24 <https://documents1.worldbank.org/curated/en/099042624131510202/pdf/P1756461ff912e09d1ac0212dff171451f6.pdf>

25 Evaluation of adoption of people-centred NCD service delivery within primary healthcare in WHO South-East Asia Region 2014–2021. New Delhi: World Health Organization, Regional Office for South-East Asia; 2024. Licence: CC BY-NC-SA 3.0 IGO.

26 <https://businessbhutan.bt/revolutionizing-bhutans-health-sector-a-closer-look-at-the-health-sector-landmarks-in-2023/#:~:text=On%20January%2030%2C%202023%2C%20the,capacity%2Dbuilding%20before%20the%20MOU>

Bhutan's disease surveillance system is at an advanced stage, capable of tracking health and burden of disease metrics; detecting, reporting and investigating notifiable diseases; and continuously collecting, collating and analysing resulting data. The Royal Centre for Disease Control (RCDC), established in 2009, is Bhutan's national focal point for disease surveillance, outbreak response and laboratory-based public health research. As a specialized agency under the Ministry of Health, the RCDC plays a critical role in tracking communicable diseases, strengthening laboratory capacity and supporting policy decisions through evidence-based data. In 2023, the Virology and Molecular Laboratory at the RCDC was recognized as a National Influenza Centre (NIC) by WHO, acknowledging its contributions to influenza surveillance and response.²⁷ However, challenges remain, including issues related to the timeliness of reporting and weak interoperability among these systems.

The Electronic Patient Information System (ePIS), currently being rolled out, is expected to address these gaps by enabling detailed, real-time and uniquely identified patient-level medical records.²⁸ In addition to strengthening data management, ePIS can also be leveraged to enhance patient safety and quality of care, ensuring alignment with Bhutan's national health policy. Despite its strengths, this system faces significant challenges.

The country is facing a serious shortage of human resources required for health-care services, as indicated in Section 2.4.3. To fill these gaps, Bhutan relies heavily on international referrals for specialized treatments, which places a financial strain on the health system. Referral costs have increased by more than 150% since 2021, highlighting the urgent need to strengthen domestic capacity for advanced care.

To address these systemic challenges, Bhutan is prioritizing health workforce development and the implementation of key health sector reforms. These include the establishment of the National Medical Service (NMS), civil service transformation initiatives and regulatory reforms for health professionals – all aimed at improving health care delivery, reducing reliance on external services, and strengthening planning and deployment of health personnel.

Complementing these efforts, Bhutan is also leveraging digital innovations such as DHIS-2 and ePIS to improve service delivery and decision-making. Additionally, the development of the Multisectoral Action Plan for the Prevention and Control of Noncommunicable Diseases (MSAPII) (2025–2030) reflects the country's commitment to integrated, cross-sectoral approaches in addressing major health burdens, particularly NCDs, through collaboration across government and community stakeholders.

Refer to Sections 8.2 and 8.3 for detailed information on environmental and climate change as well as on digital health and innovation.

27 <https://www.who.int/news/item/14-07-2023-recognition-of-bhutan-s-national-reference-laboratory-as-a-national-influenza-centre-in-the-who-global-influenza-surveillance-and-response-system>

28 World Bank (2023). Bhutan health sector review: Advancing health equity and system resilience. Retrieved from <https://documents1.worldbank.org/curated/en/099042624133516182/pdf/P17054414014ae0b61a154156f3f80a11f7.pdf>.

3. Partnerships and collaborations

3.1 History of WHO in Bhutan

Forged in 1982, WHO's partnership with Bhutan has supported and achieved critical milestones such as the elimination of polio and measles, advancing maternal and child health, and the development of universal health coverage (UHC) principles. The WHO Country Office in Bhutan plays a pivotal role in supporting the Ministry of Health (MoH) in disease prevention, health promotion, rehabilitation, protecting the health of the vulnerable population and building a resilient health system in the country. Guided by WHO's global mandate, the Country Office provides technical support, fosters partnerships, and strengthens institutional capacity to advance Bhutan's health priorities in alignment with national, regional and global health strategies.

Acting as the lead technical partner to the Ministry of Health, WHO leverages its convening and coordination role; through its numerous WHO collaborating centres and regional offices, it brings in the technical expertise needed to build national capacities for improving health services in the country. For WHO's collaborative history with Bhutan, demonstrating the progression and impact of this long-standing partnership, refer to the Appendices Section 8.4.

3.2 Development partners

WHO's collaboration in Bhutan is enriched and strengthened through partnerships with a range of development partners, including bilateral and multilateral agencies, global health initiatives and regional organizations. There are other bilateral and multilateral development partners that contribute to the development of health in Bhutan. Among bilateral governments, partners and collaborators are the Government of India, Thailand, Japan and Bangladesh. Key development partners in Bhutan include:

Gavi, the Vaccine Alliance	Supporting Bhutan's immunization programmes, leading to the elimination of vaccine-preventable diseases such as measles and rubella
Global Fund	Providing technical and financial support for combatting tuberculosis, malaria and HIV/AIDS, along with efforts to address antimicrobial resistance (AMR)
Asian Development Bank (ADB)	Facilitating investments and health financing in health-related infrastructure and climate-resilient health systems, along with governance efforts
Bilateral partners	Countries such as India and Japan have contributed to the development of Bhutan's health infrastructure and capacity-building.
World Bank	Supporting health systems strengthening, health financing, improving health governance and addressing social determinants of health
Other international organizations	Organizations such as Japan International Cooperation Agency (JICA) have played a critical role in addressing broader determinants of health through financial and technical support. The Fleming Fund has strengthened Bhutan's AMR surveillance and laboratory capacity under the National Action Plan on AMR.

3.3 Collaboration with UN partners at the country Level

WHO's collaborative work in Bhutan is strengthened by its partnerships with UN agencies and development partners. The United Nations Sustainable Development Cooperation Framework (UNSDCF) 2024–2028, which aligns with Bhutan's 13th Five-Year Plan and global health priorities, outlines a unified approach among UN entities to support Bhutan's 13th Five-Year Plan and its vision of becoming a high-income, sustainable and resilient country by 2034. The UNSDCF is structured around four Strategic Priorities that guide joint UN efforts to support the development aspirations of Bhutan:

- (1) **Sustainable economic development and diversification, and shared prosperity:** By 2028, Bhutan achieves sustainable and diversified economic growth, decent employment, livelihoods and shared prosperity.
- (2) **Equitable human development and well-being across the life-course:** By 2028, people in Bhutan benefit from strengthened quality of life, and inclusive and lifelong social services and practices. This is where WHO makes its primary contribution to the UNSCDF.
- (3) **Sustainable and healthy environment, resilience to disasters and climate change:** By 2028, Bhutan's environment remains sustainably managed, and its people are more resilient to disaster risks and climate change.
- (4) **Inclusive, transparent and accountable governance, and rule of law:** By 2028, Bhutan has more inclusive, transparent and accountable governance, and rule of law.

These Priorities emphasize a whole-of-society approach, with a focus on leaving no one behind and promoting gender equality, disability inclusion and resilience. WHO contributes to these outcomes through its technical expertise in health, capacity-building and policy development. WHO collaborates with the following UN agencies in Bhutan:

United Nations Children's Fund (UNICEF)	Joint efforts in immunization, maternal and child health, and nutrition programmes, supporting school health including education, adolescent well-being, water, sanitation and hygiene (WASH), and violence against children
United Nations Population Fund (UNFPA)	Supporting Bhutan's reproductive and adolescent health initiatives and addressing gender-based health inequities
United Nations Development Programme (UNDP)	Collaborating on health systems strengthening, emergency preparedness and addressing social determinants of health
United Nations Educational, Scientific and Cultural Organization (UNESCO)	Promoting health literacy and incorporating health-focused curriculums into Bhutan's education system, transformative education for health and well-being as part of Education 2030
World Food Programme (WFP)	Supporting nutrition security

3.4 Bhutan's contribution to regional and global health agenda

Bhutan actively engages in WHO governing body sessions, including the World Health Assembly and the Regional Committee for South-East Asia, where critical global and regional health issues are discussed and resolutions adopted for implementation by Member States. Demonstrating its leadership and commitment to regional public health, Bhutan hosted the Seventy-fifth Session of the WHO Regional Committee for South-East Asia in Paro in September 2022, providing a platform to shape and advance the regional health agenda. Bhutan has also contributed to the South-East Asia Primary Health Care Forum and continues to advocate for equity, resilience and sustainability in health systems.

Bhutan has played a proactive role in advancing health priorities within these forums, advocating for equity, sustainability and resilience in health systems. For example, Bhutan has contributed to the discussions on the Global Health Sector Strategy on HIV, viral hepatitis and sexually transmitted infections, aligning its national strategies with global and regional frameworks. It has demonstrated leadership in health emergency preparedness and response, actively participating in regional initiatives under the South-East Asia Regional Health Emergency Fund (SEARHEF) to enhance collective health security.

Furthermore, Bhutan aligns with WHO's climate change and health initiatives, advocating for the integration of climate resilience into health systems across the Region. As a member of the South Asian Association for Regional Cooperation (SAARC), Bhutan collaborates on addressing regional health challenges, such as communicable diseases, NCDs and health workforce development. The country has made significant contributions to achieving regional health targets, such as the elimination of measles and rubella, and the progress towards cervical cancer elimination. Additionally, Bhutan has prioritized advancing traditional medicine at the global level, sharing its experiences and best practices in integrating traditional medicine into modern health-care systems. Bhutan's leadership in this area underscores its commitment to contributing to culturally sensitive and holistic approaches to health.

Through its active participation in regional and global health forums, Bhutan continues to strengthen its role as a key advocate for universal health coverage, equitable health care access and sustainable development. Notably, Ms Dechen Wangmo, former Minister of Health, served as a member of the Global Leaders Group (GLG) on Antimicrobial Resistance, a high-level body comprising global leaders and experts advocating political action on AMR. This involvement not only reinforces Bhutan's commitment to its national health goals but also contributes to the collective efforts to achieve global health priorities.

Impact of partnerships

The collaborative work of WHO with UN agencies and development partners has been instrumental in achieving key health milestones:

- Bhutan became one of the first countries to meet the interim 2030 targets for cervical cancer elimination.
- High routine immunization coverage was sustained, contributing to the elimination of rubella and measles.

- Significant strides were made in combatting communicable diseases and enhancing resilience to health emergencies.
- The country has significantly reduced maternal and child mortality, and is on track to achieving SDG 3.

Additionally, in collaboration with partners such as the UNDP and the Government of Japan, WHO has supported Bhutan in strengthening its health-care infrastructure, including the provision of specialized medical waste collection trucks. These efforts are vital in managing infectious waste, promoting environmental safety and safeguarding public health.²⁹ Collaborative projects such as improving infectious waste management have created sustainable solutions, directly impacting public health and environmental safety.³⁰

29 <https://www.undp.org/bhutan/news/japan-and-undp-support-5-specialised-medical-waste-collection-trucks-health-ministry>

30 <https://www.undp.org/bhutan/projects/project-improvement-infectious-waste-management>

4. Key evaluation findings: Reflections on CCS 2020–2025

This section has been derived from the evaluation of Bhutan’s Country Cooperation Strategy (CCS) 2020–2025, which assessed WHO’s contributions across its three levels, namely Country Office, Regional Office and headquarters, in achieving national health priorities. Conducted between January and March 2025, the independent evaluation applied WHO’s criteria of relevance, coherence, effectiveness, efficiency and sustainability through stakeholder interviews, desk reviews and health system data analysis.

WHO’s leadership, technical expertise and partnerships have been key to Bhutan’s progress in disease elimination, maternal and child health, WASH at health-care facilities and high immunization coverage. However, challenges persist, including fragmented and unreliable funding, limited engagement with other stakeholders beyond the Ministry of Health, gaps in gender and equity strategies, and to a certain extent, unrealistic expectations of support. Addressing these requires efficiency measures, stronger multisectoral collaboration, and expanded partnerships with multilateral development banks (MDBs), UN agencies and civil society.

Future efforts should focus on implementing the next phase of the NCD Multisectoral Action Plan, integrating traditional medicine, strengthening community-driven nutrition, and exploring new horizons of addressing the root causes of the rising NCD burden through food and nutrition advocacy.

4.1 Summary of CCS 2020–2025

The CCS 2020–2025 period was strategically aligned with Bhutan’s 12th Five-Year Plan (FYP), the WHO Thirteenth Global Programme of Work 13 (GPW 13) and the Sustainable Development Goals (SDGs). The Strategy was designed around five Strategic Priorities:

- (1) Achieve UHC through integrated, people-centred, quality health-care services.**
- (2) Improve reproductive, maternal, newborn, child and adolescent health (RMNCAH), and promote healthy ageing.**
- (3) Build health system resilience to address communicable diseases and effects of health emergencies and climate change.**
- (4) Address NCDs and their determinants with Strategic Priority.**
- (5) Strengthen data, research, innovation and knowledge brokerage.**

The CCS 2020–2025 focused on advancing universal health coverage (UHC); addressing reproductive, maternal, newborn, child and adolescent health (RMNCAH); building resilience to communicable diseases and health emergencies; tackling the burden of noncommunicable diseases (NCDs); and fostering innovation in data and research. These Priorities reflected WHO’s long-standing commitment to strengthening Bhutan’s health system, improving health outcomes and ensuring equity in service delivery.

4.2 CCS 2020–2025: Key achievements

Below are the key achievements associated with each Strategic Priority during the CCS 2020–2025 period:

Among the first countries in the WHO South-East Asia Region to achieve the elimination of rubella (as of July 2023) achieving the regional target of eliminating measles and rubella. This reflected Bhutan's strong commitment to public health and the success of its vaccination programmes.	Significant reduction in Infant mortality rate and Under five mortality rate to 15.2 and 19.5 respectively in 2023. Additionally, the country achieved and maintained high vaccination coverage ³¹ at 99.4% for measles and rubella for children aged 12-23 months.	Expanded the implementation of the PEN package in the remaining 6 districts, successfully achieving coverage in all 20 districts. Developed a multisectoral action plan aligned with Bhutan's evolving health needs. ³²
Bhutan achieved the interim target for cervical cancer elimination, marking a significant milestone in its commitment to women's health.	The Integrated National Strategy on RMNCAH 2024 was developed to strengthen reproductive, maternal, newborn, child, and adolescent health services.	Installation and repair of open gym equipment in key locations to promote physical activity and well-being.
National Health Survey 2023 provided updated health indicators to guide policymaking and program implementation	Recognised and awarded in September 2022 for successful implementation of the Service with Care and Compassion initiative (SCCI) at district level in Bhutan. ³³	Rolled out and expanded WASH FIT initiative to all 20 districts of Bhutan with the partnership of UNICEF and SNV Bhutan. ³⁴
WHO supported the National Health Accounts (NHA) studies—aligned with WHO's System of Health Accounts (SHA 2011)—for the fiscal years 2020–21, 2021–22, and 2022–23	Introduced the Bhutan Vaccine System (BVS) to digitally track vaccination data and adverse events following immunization, ensuring equitable distribution, particularly to remote areas via helicopters.	Strengthened Bhutan's mental health system with the establishment of the PEMA Secretariat, under Her Majesty the Gyaltsuen's guidance, to streamline and harmonise all national mental health interventions.
WHO supported the Ministry of Health to conduct Bhutan's first ever National Blood Lead Level Survey in 2024. The survey aimed to determine lead prevalence and identify sources to guide public health interventions in Bhutan.	Swift and effective COVID-19 response, achieving one of the fastest vaccination campaigns globally, with over 90% coverage in just weeks. WHO provided \$7 million in funding, two million PPEs, and trained 15,000+ frontline workers to support Bhutan's pandemic response.	WHO supported a nationwide NCD screening integrating early detection for hypertension, diabetes, and other risk factors. Engaged monks and media in NCD advocacy, reinforcing the role of community-based interventions in prevention efforts

31 5th National Health Survey 2023

32 World Health Organization (2023). Output Scorecard Tool: Assessment for SE_BHU WR Office, Bhutan – Output: Countries enabled to strengthen their health systems to address population-specific health needs and barriers to equity across the life course (1.1.2), 2022–2023 – End of Biennium. Geneva, Switzerland.

33 <https://www.who.int/news/item/21-09-2022-2022-united-nations-inter-agency>

34 Assessment for SE_BHU WR Office, Bhutan – Output: Countries enabled to address risk factors through multisectoral actions (3.2.1) 2022–2023 – End of Biennium

Bhutan's new national strategic plan for TB developed, providing a roadmap toward the pre-elimination of TB in alignment with global health targets ³⁵	Expanded mental health services by integrating mental health into primary healthcare (PHC) and scaling up community-based mental health programs	Supported the implementation of national flagship project - electronic patient information system (ePIS) in 2023 ³⁶
WHO supported the Ministry of Health in training nurses, doctors and managers on Infection Prevention & Control (IPC) tailored to country contexts.	WHO supported a comprehensive review of the National Medical Services (NMS) and Jigme Dorji Wangchuck National Referral Hospital (JDWNRH), focusing on governance, infrastructure, human resources, and management systems. The review aimed to position JDWNRH as a model for high-quality, patient-centric care. One of the key recommendations was to conduct a similar bottom-up review of the primary healthcare (PHC) system to strengthen PHC orientation and ensure integrated, equitable service delivery across all levels of the health system.	

4.3 Challenges and lessons learnt

Building on the achievements under the previous CCS period, Bhutan's journey in improving health outcomes also offers valuable lessons that will inform not only future collaborations but also opportunities. These lessons, coupled with emerging opportunities, provide a strategic pathway for the WHO CCS 2026–2030 to address challenges and leverage strengths effectively.

 The importance of health workforce development:	Bhutan's health-care system has benefited from WHO-supported initiatives. Its efforts in expanding MD speciality programmes and launching its first MBBS programme highlight the critical need for continuous investments in workforce development. However, retaining skilled professionals in rural areas and addressing gaps in specialized health-care services remain ongoing challenges. The successful deployment of targeted training and fellowship programmes under WHO support offers a replicable model for capacity-building. Despite recent investments in domestic medical training, Bhutan now faces the challenge of domestic health worker migration. Additionally, enhancing the quality of care will require a robust competency assessment framework, strengthened human resources for health (HRH) information systems and improved regulatory capacity of institutions, such as the Bhutan Medical and Health Professions Council, to ensure quality assurance, accreditation and professional standards across the health workforce.
 NCDs as a persistent burden:	NCDs account for 72% of deaths in Bhutan, driven by high-risk behaviours such as tobacco use, alcohol abuse, poor nutrition and physical inactivity. The rising mental health crisis, including increasing suicide rates, exacerbates this challenge. Despite existing efforts, integrated multisectoral approaches involving sectors such as nutrition, agriculture and civil society remain underdeveloped. Collaborative initiatives, such as engaging monastic bodies and finalizing the Multisectoral Action Plan for NCD Prevention and Control, offer valuable lessons on addressing these systemic issues.

35 World Health Organization (2023). Output Scorecard Tool: Assessment for SE_BHU WR Office, Bhutan – Output: Countries enabled to strengthen their health systems to address population-specific health needs and barriers to equity across the life course (1.1.2), 2022–2023 – End of Biennium. Geneva, Switzerland.

36 Assessment for SE_BHU WR Office, Bhutan – Output: Countries enabled to strengthen data, analytics and health information systems to inform policy and deliver impacts. (4.1.1) 2022–2023 – End of Biennium.

 Need for stronger coordination mechanisms to handle public health emergencies:	Despite Bhutan’s exemplary response in managing the COVID-19 pandemic, the crisis also highlighted lack of integration between the Disaster Risk Management Strategy and the Strategy Plan for Emergency Medical Services of the Ministry of Health, and key international commitments such as the International Health Regulations (2005) [IHR (2005)]. In the post-COVID-19 era, Bhutan must adopt a multihazard approach by revising preparedness strategies, ensuring collaboration between health and disaster agencies, and conducting regular evaluations and lessons-learned exercises.³⁷
 Funding mechanisms for health emergencies and disasters:	The COVID-19 pandemic also revealed the lack of sustainable funding mechanisms for public health emergencies that led the country to relying on reprogramming budgets and external borrowing during crises. Establishment of funding mechanisms with sufficient resources and development of financing strategies to expedite resource mobilization during emergencies while minimizing disruptions in ongoing health programmes are paramount to ensure timely and sustainable financing for preparedness and responses.
 Building infrastructure resilience for health emergencies:	Investments in isolation wards, critical care units and facility-wide infection prevention and control (IPC) measures have proven valuable in strengthening Bhutan’s preparedness for future pandemics.

4.4 Opportunities

Advancing the Thrive and Transform Agenda:	With progress in reducing maternal, child and adolescent mortality, Bhutan can now shift its focus from survival to overall well-being, aligning with the Thrive and Transform Agenda under the Global Strategy for Women’s, Children’s and Adolescents’ Health (2016–2030). This approach emphasizes mental health, nutrition, early childhood development and adolescent well-being, integrating these into primary health care and education systems. A multisectoral strategy involving education, gender equity and social protection will strengthen resilience and long-term health outcomes. Leveraging health-promoting schools, community programmes and policy leadership from the Prime Minister’s Office and the Royal Family will ensure sustainable health interventions, reinforcing Bhutan’s UHC goals and SDG commitments.
Unrealized benefits of ICT:	One of the key challenges is the unrealized benefits of information and communication technology in supporting patients’ self-management of their illnesses by facilitating the exchange of health information between patients and health-care providers, i.e. through m-health technologies, personal health records and other digital solutions.

³⁷
pdf?

<https://www.bhutanaudit.gov.bt/wp-content/uploads/2024/07/Strong-Resilient-Health-System-linked-to-SDG-3.d-12.07.2024.pdf>

Enhancing multisectoral collaboration:	The preparation of Bhutan's Multisectoral Action Plan for NCD Prevention and Control provides a strategic opportunity to strengthen intersectoral coordination. Leveraging partnerships with civil society, including monastic bodies, and integrating lessons from regional and global good practices can catalyse these efforts. There is a need for multisectoral advocacy to strengthen advocacy efforts involving schools, religious institutions, parents and other community stakeholders to promote healthy behaviours. Bhutan's National Health Promotion Strategic Plan (NHPSP) 2015–2023 emphasizes a whole-school approach to health, integrating nutrition, physical activity and mental well-being into the education system. The health-promoting schools initiative aligns with this strategy by creating safe and supportive environments that encourage healthy behaviours among students and staff, ensuring that schools play a central role in disease prevention and health promotion. Collaborating with the agriculture sector is essential to ensure accessible and sustainable local healthy food products, fostering sustainable dietary habits and integrating healthy eating practices. Strengthen community-based healthy productions and regulate import of unhealthy products (including importation of chemical-contaminated food products and other substances). Bolster collaboration with the Prime Minister's Office and the Royal Family to advocate for and prioritize key sustainable dietary habits and health initiatives.
Strengthening the One Health approach:	Strengthen Bhutan's One Health framework by fostering collaboration among key sectors, including human health, animal health and environmental health. Partner with the Food and Agriculture Organization of the United Nations (FAO), the World Organisation for Animal Health (WOAH) and the World Health Organization (WHO) to address zoonotic diseases, antimicrobial resistance and climate-sensitive health risks. Integrate surveillance systems, promote intersectoral training and develop strategies to mitigate health risks at the human–animal–environment interface. This holistic approach will enhance Bhutan's capacity to respond to emerging and re-emerging health threats.
Sustaining UHC through innovative financing:	Bhutan's transition from the least developed country (LDC) status calls for innovative health financing models. Exploring insurance schemes, engaging private sector partnerships and leveraging global health funds can ensure the sustainability of UHC without compromising on quality or equity. Strategic purchasing mechanisms can optimize resource allocation by ensuring that funds are directed towards high-impact, cost-effective interventions. Improving service delivery models – through technology integration, workforce planning and primary health care strengthening – is not only essential for enhancing access and quality, but it is also a critical strategy for addressing equity gaps. These reforms are equally important for optimizing financing and ensuring that health investments translate into inclusive, long-term impact.
Strengthening climate-resilient health systems:	As indicated in <u>Bhutan's National Adaptation Plan</u>, the country has significant opportunities to strengthen climate-resilient health systems, leveraging its commitment to sustainability and emphasis on environmental conservation that provide a strong foundation to incorporate climate resilience into health policies. The Ministry of Health, in coordination with other ministries and local governments, could consider integrating climate-related risks into the National Health Strategy to address vulnerabilities such as heatwaves, floods and landslides. A multisectoral approach, involving the Department of Local Governance and Disaster Management under the Ministry of Home Affairs, is essential to build preparedness and response systems across sectors. Expanding disease surveillance to track climate-sensitive and zoonotic diseases will enhance early detection and risk mitigation. Strengthening an integrated early warning system will improve emergency preparedness and evidence-based decision-making, ensuring Bhutan's health system remains resilient to climate challenges.

Institutionalizing health technology assessment (HTA):	Bhutan's recently developed 2023 HTA framework provides an opportunity to streamline decision-making, prioritize resource allocation and improve cost-effectiveness in health interventions. It also provides an opportunity to address public health emergencies faster and more effectively through the newly added expedited/rapid HTA process for public health emergencies.
Leveraging the success model for mental health care:	Bhutan's Service with Care and Compassion Initiative (SCCI) that aims to provide comprehensive NCD services at the primary health care level has been a successful model; it offers the opportunity to scale up preventive and curative services, including mental health care. Expanding this model beyond treatment to a broader health and well-being approach aligns with the Thrive and Transform Agenda under the Global Strategy for Women's, Children's and Adolescents' Health. This shift emphasizes not only addressing illnesses but also fostering resilience, promoting healthy lifestyles and ensuring overall well-being. Additionally, <i>Findings from the Primary Health Care Performance Initiative Assessment</i> also recommends integration of supportive supervision and task-shifting through the SCCI programme.
Expanding and strengthening digital health systems:	Further strengthening Bhutan's digital health systems integration of data platforms, such as Civil Registration and Vital Statistics (CRVS), District Health Information System 2 (DHIS-2) and Electronic Patient Information System (ePIS), would allow for enhanced performance monitoring and accountability and informed decision-making on health at multiple levels.³⁸ Ensuring interoperability across these platforms – by adopting robust and open standards such as Fast Healthcare Interoperability Resources (FHIR), as emphasized in Bhutan's draft National Digital Health Strategy – will be critical for enabling seamless data exchange and system efficiency. Furthermore, the nationwide rollout of International Classification of Diseases 11th Revision (ICD-11) would standardize disease classification, ensuring better health data management and reporting. Expanding telemedicine services can increase health care accessibility, particularly in remote areas, while promoting e-health literacy ensures that individuals can effectively engage with digital health tools. Incorporating health-promotive devices and mobile applications can support preventive care, self-monitoring and chronic disease management, reinforcing Bhutan's commitment to digital health transformation.
Learning to apply the equity lens:	While Bhutan has made significant strides, there is an opportunity to strengthen the efforts from an equity lens to address gaps in quality access and outcomes in underserved pockets. A focused approach to ensure inclusive benefits for all communities will strengthen future interventions.
Leadership to strengthen mental health initiatives:	Strong leadership, stewardship and guidance from Her Majesty the Gyaltsuen Jetsun Pema Wangchuck have been instrumental in strengthening and harmonizing mental health interventions across relevant agencies in Bhutan, ensuring a cohesive and well-coordinated national response.

4.5 Key recommendations for CCS 2026–2030

The key recommendations from the independent evaluation as part of the Country Programme Evaluation-Country Cooperation Strategy development exercise (the first of its type commissioned by WHO) are summarized below:

38 <https://documents1.worldbank.org/curated/en/099042624133516182/pdf/P17054414014ae0b61a154156f3f80a11f7.pdf>

Recommendation 1: Ensure continuity between CCS 2020–2025 and CCS 2026–2030.

- ◉ Focus on NCDs, strengthen primary health care (PHC) and help ensure that the NCD Multisector Action Plan II (MSAPII) is activated and well-coordinated.
- ◉ Continue to support Bhutan in efforts to eliminate malaria, reduce HIV and STIs, and eliminate cervical cancer.
- ◉ Close gaps in emergency response capacity related to the IHR (2005) Framework and establish a new National Action Plan for Health Security.
- ◉ Pay greater attention to health-related consequences of climate change.

Recommendation 2: Strengthen cooperation and partnership to address Bhutan's key health challenges.

The WHO Country Office in Bhutan should:

- ◉ exercise its convening power to create collective multisectoral coordination addressing NCD drivers, in close cooperation with the government and aligned with the 13th FYP for Health;
- ◉ continue supporting efforts to address the crisis in health staffing and retention;
- ◉ renegotiate with the MoH the use of its core funding to make it more strategic by reducing the number of Direct Financial Cooperations (DFCs) and targeting resources to priority interventions; and
- ◉ strengthen partnerships with monastic institutions and traditional medicine to expand public communication on healthy living.

Recommendation 3: Improve equitable access to health services.

The WHO Country Office in Bhutan should:

- ◉ strengthen its work on understanding gendered inequalities in health, identifying health access barriers for women and girls, and develop strategies to overcome them;
- ◉ work with the MoH to enable data exchange between National EWARS, DHIS-2-based HMIS and ePIS to reduce fragmentation among health databases; and
- ◉ engage local CSOs to maximize their expertise and understanding of local-level realities of people living with HIV, cancer and disability.

Recommendation 4: Improve budget management, performance monitoring and learning.

The WHO Country Office in Bhutan should:

- ◉ assign the majority share of its budget at the start of the year (ideally, the biennium) in a limited number of relatively large DFCs aimed at Strategic Priorities within the CCS; and
- ◉ assign more resources for evaluating the impact of its major interventions in Bhutan, with technical support from the WHO Regional Office for South-East Asia (WHO-SEARO).

WHO-SEARO should:

- define quality standards for its advice and advocacy functions, monitor the appropriateness of its recommendations and periodically review the level of implementation by national partners; and
- organize learning meetings to facilitate knowledge-sharing between countries of the Region on issues raised during the evaluation, including best practices in multisectoral coordination for NCDs, performance monitoring, strategic resource allocation and other topics of common interest.

WHO headquarters (HQ) should:

- devise tools for the periodic assessment of the performance of both country offices and regional offices, using both self- and independent assessment to close the current gap in evaluative data.

5. Strategic Priorities

5.1 Strategic Priorities, focus areas and deliverables

The Country Cooperation Strategy (CCS) 2026–2030 builds on Bhutan’s evolving health landscape and aligns with global health priorities to strengthen health systems, address emerging issues and leverage opportunities. The Strategic Priorities were identified through a consultative and participatory process, including an independent evaluation of the previous CCS 2020–2025, an extensive desk review of WHO Fourteenth General Programme of Work (GPW 14, 2025–2028), Bhutan’s 13th Five-Year Plan (2024–2029) and the UN Sustainable Development Cooperation Framework (UNSDCF) for Bhutan (2024–2028). This process also included participatory consultations with key stakeholders – the Ministry of Health, WHO technical teams, UN agencies, development partners, civil society and academia – ensuring a comprehensive and inclusive approach. The Strategic Priorities were iteratively refined based on stakeholder feedback, ensuring alignment with the health goals of Bhutan and the overarching objectives of WHO. A list of stakeholders interviewed can be found in the Appendices Section 8.5.

These Strategic Priorities align with Bhutan’s commitment to achieving equitable and sustainable health care and have been identified through structured prioritization under the GPW14 for the programme year 2026–2027. Each country in the WHO South-East Asia Region follows a collaborative prioritization process with ministries of health and key stakeholders to ensure regional coherence. The Regional Priorities, introduced in 2024, emphasize cross-cutting and integrated approaches across all programme areas to address both health systems strengthening and emerging health threats.

This CCS will form the basis for preparing the WHO biennial work plans during 2026–2030, articulating how the effectiveness, efficiency and quality of WHO’s work in Bhutan will be enhanced. The goal is to ensure the greatest possible contribution to health and development outcomes while addressing Bhutan’s specific needs. To ensure the successful implementation of these Strategic Priorities, Bhutan will ***emphasize efficient financial resource utilization, ensuring cost-cutting measures, and leverage existing resources and improved resource allocation to maximize the impact of health investments***. Achieving these objectives will require strong cross-sectoral collaboration and partnerships to address Bhutan’s evolving health challenges holistically.

The **key draft Strategic Priorities identified include:**

- (1) **Expand multisectoral collaboration to address NCDs and mental health across life-course:** Strengthen the prevention and management of NCDs, mental health and nutrition-related disorders through multisectoral engagement. This includes scaling up hypertension and diabetes management, expanding HPV vaccination and cancer screening, and promoting mental health awareness and community-based services. There will be a special focus on nutritional literacy, gut-brain health and dietary interventions to reduce NCD risks.
- (2) **Prevention and control of communicable diseases:** Strengthen surveillance, prevention and treatment efforts for HIV, TB, malaria, vaccine-preventable diseases (VPDs), and emerging and re-emerging infections. Support and strengthen the National Tuberculosis Programme and the implementation of the National Action Plan on AMR (2024–2028).

Launch stigma-reduction campaigns for HIV/STIs, and improve digital disease surveillance and vector control to achieve targets for malaria elimination and other NTDs such as kala-azar, rabies and leprosy.

- (3) **Achieve equitable universal health coverage (UHC):** Review and strengthen primary health care, immunization and SRMNCAH services to ensure accessible and high-quality care for all. Enhance health workforce competence, distribution and performance through focused training, career development and regulatory capacity-building. Expand digital health services and develop a roadmap for sustainable health financing.
- (4) **Strengthen emergency preparedness and response:** Enhance health system resilience by updating Bhutan's National Health Disaster Plan, strengthening IHR (2005) Core Capacities and ensuring equitable access to essential services during emergencies. Train health-care workers in outbreak management, field epidemiology and health information systems while fostering intersectoral collaboration and capacity enhancement for zoonotic and emerging disease prevention under the One Health framework.

5.1.1 Strategic Priorities, focus areas and deliverables

Strategic Priority	Focus areas	Deliverables
 Expand multisectoral collaboration to address NCDs and mental health across life-course.	1. NCD and risk factors management and control 2. Cancer 3. Mental health	1.1 Support the Ministry of Health, Bhutan, in preventing, promoting awareness of and managing NCDs while expanding efforts to address risk factors. 1.2 Support the development and implementation of the National Multisectoral Action Plan for NCD Prevention and Control (MSAPII). 1.3 Expand the implementation of WHO NCD Best Buys tailored across various life stages. 2.1 Sustain and strengthen HPV vaccination coverage and integrate cervical cancer screening into primary health care. 2.2 Support the scale-up of cervical, gastric and breast cancer screening programmes, with enhanced access to HPV DNA testing, endoscopy services and mammography, covering underserved populations. 3.1 Support the Mental Health Gap Action Programme (MhGAP), LIVE LIFE (WHO technical package for suicide prevention) and any other technical packages for suicide prevention.
 Prevention and control of communicable diseases	1. Malaria prevention and control including other NTDs 2. HIV/STI 3. Tuberculosis 4. Infectious diseases prevention and control	1.1 Sustain and strengthen Bhutan's malaria elimination efforts and control programmes for neglected tropical diseases such as kala-azar, leprosy and rabies to achieve and maintain elimination. 2.1 Support and strengthen the National STI and HIV/AIDS Control Programme. 3.1 Support and strengthen the National Tuberculosis Control Programme. 4.1 Strengthen systems for infectious disease prevention and control including supporting the implementation of the National Action Plan on AMR (2024–2028).

Strategic Priority	Focus areas	Deliverables
 Achieve equitable universal health coverage (UHC).	1. Comprehensive primary health care and immunization 2. Digital health and telemedicine 3. Human resources for health 4. Sustainable health financing	1.1. Support and strengthen the SRMNCAH Strategy 2024–2029 using a life-course approach. 1.2 Strengthen the National Immunization Strategy (NIS) 2024–2028. 1.3 Review and strengthen PHC orientation of health systems. 1.4 Achieve Maturity Level 2 for medicines and vaccines regulatory system and Maturity Level 3 in priority domains against the global benchmarking tool. 2.1 Support the implementation of the National Digital Health Strategy, including ePIS and other digital health platforms. 3.1 Support the development and implementation of Bhutan’s HRH strategy and strengthen Human Resources Information System (HRIS) to address workforce gaps and retention issues. 3.2 Sustain capacity-building programmes and mentorship initiatives for HRH on clinical and non-clinical priority topics, strengthen regulatory capacity of the Bhutan Medical and Health Professions Council. 4.1 Update and finalize the Health Financing Progress Matrix. 4.2 Establish a strategic roadmap for health financing and build capacity in health expenditure tracking. 4.3 Produce periodic National Health Accounts and financial protection estimates.
 Strengthen emergency preparedness and response.	1. Health emergency prevention and preparedness 2. Zoonotic and emerging diseases prevention and preparedness	1.1 Develop and implement guidelines and SOPs to ensure continuity and equitable access to essential health services during large-scale emergencies. 1.2 Support NAPHS development and implementation based on JEE and IHR (2005) review findings, with emphasis on institutional capacity-strengthening for emergency preparedness. 2.1 Institutionalize multisource collaborative surveillance systems, integrating data from different sectors and sources. 2.2 Foster intersectoral collaboration and capacity enhancement to strengthen preparedness for zoonotic and emerging diseases under the One Health framework.

5.2 Theory of change

If Bhutan’s CCS 2026–2030 is effectively implemented, then its health system will be significantly strengthened through addressing key structural barriers, including health workforce shortages, geographical and infrastructure challenges, the growing burden of noncommunicable diseases (NCDs), limited specialized health-care capacity, and emerging health threats such as climate change and antimicrobial resistance.

WHO’s technical assistance, policy guidance, financing and strategic partnerships will serve as the foundation for this transformation. If WHO provides targeted support in governance reforms, evidence-based decision-making and institutional capacity-building, then Bhutan will be able to strengthen health system management, improve policy implementation and enhance resource mobilization. This would enable the government to allocate resources more effectively, improve health infrastructure and expand access to essential services.

If these structural improvements occur, Bhutan will be well-positioned to implement key strategic deliverables. These include expanding access to NCD and mental health services, strengthening

communicable disease prevention and control, building a more resilient health-care workforce and improving emergency preparedness. For example, better-trained health workers and improved infrastructure in rural areas will lead to earlier diagnosis and treatment of NCDs and mental health conditions while enhanced disease surveillance and early warning systems will allow for quicker identification and response to public health emergencies.

If these strategic deliverables are effectively rolled out, Bhutan will experience a set of critical outcomes. The disease burden will decrease as prevention and treatment improve. Emergency response times will be shorter, reducing mortality and complications during health crises. Workforce retention will improve through better incentives and professional development opportunities, leading to more consistent and higher-quality care. Bhutan's health system will also become more adaptive and resilient, ensuring that health-care services reach even the most remote and vulnerable communities.

If these outcomes are achieved, Bhutan will experience a broader impact: improved health and well-being for all Bhutanese people through more equitable access to quality health care, stronger disease prevention and enhanced resilience against emerging health threats. The health system will become more sustainable, with greater local capacity to respond to health challenges without any over-reliance on external support.

Provided that the following assumptions hold true:

- Political commitment to health systems strengthening remains stable.
- Adequate financial resources are secured and allocated effectively.
- Bhutan maintains strong partnerships with WHO and other development partners.
- Community engagement and trust in the health system continue to grow.

However, there are key risks that can threaten success:

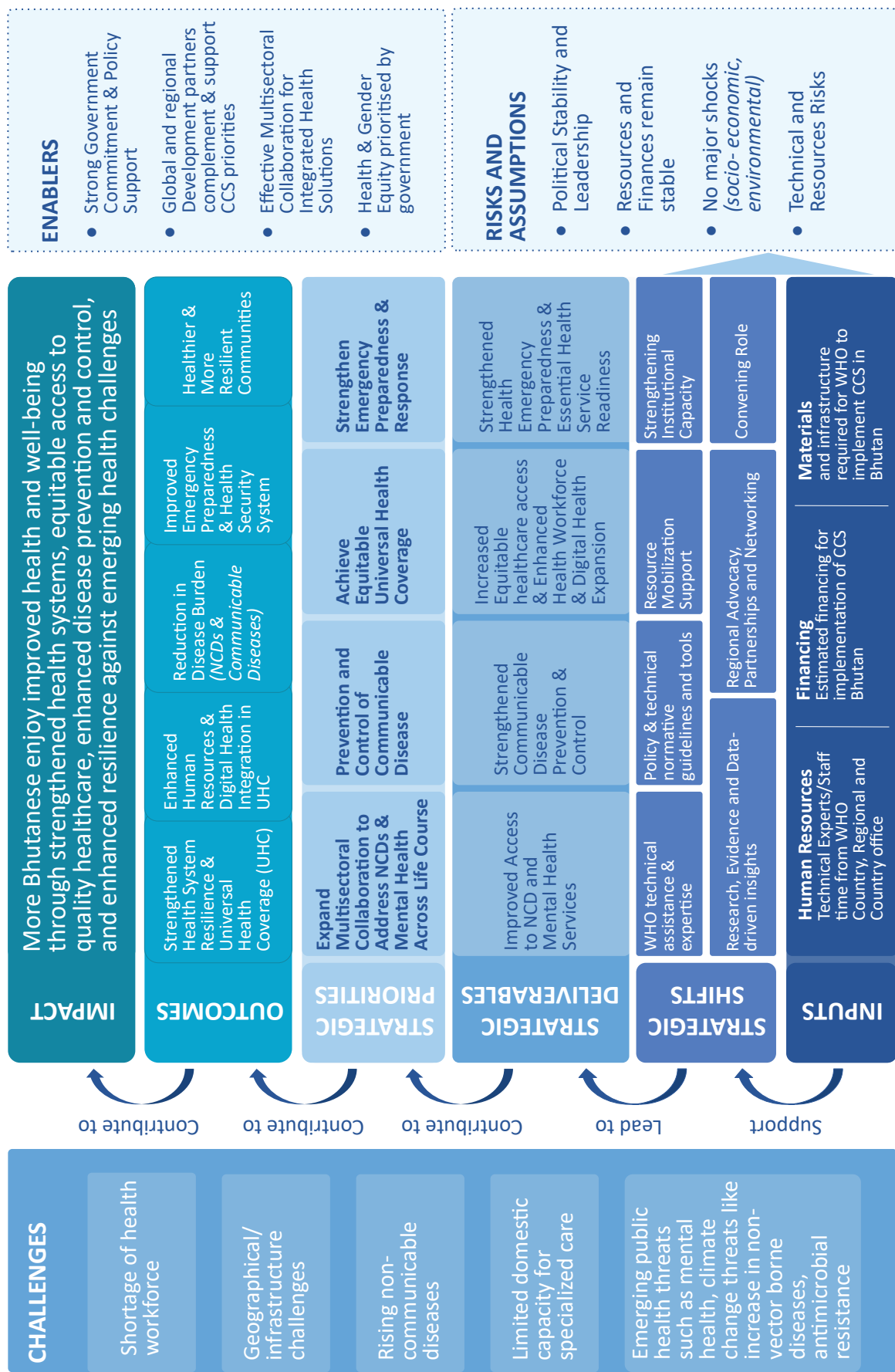
- Political instability could disrupt health governance and funding priorities.
- Resource constraints may limit the reach and effectiveness of health interventions.
- Climate-related disruptions (e.g. floods, landslides) could damage health infrastructure and interrupt service delivery.
- Health worker migration could undermine workforce capacity.

These risks can be mitigated through:

- institutionalizing financial sustainability mechanisms to reduce dependency on external funding and ensure stable resource allocation;
- strengthening governance structures and multisectoral coordination to ensure cohesive planning and response mechanisms;
- expanding early warning systems and digital health innovations to improve disease surveillance and timely intervention;
- enhancing workforce retention strategies through improved incentives, professional development programmes and decentralized service delivery models; and
- fostering local partnerships and community engagement to ensure service continuity in remote and vulnerable areas.

By strategically addressing these risks and maintaining strong governance and financial sustainability, Bhutan can achieve a more resilient, equitable health system that protects and promotes the health and well-being of all its citizens.

5.2.1 Theory of change for the four Strategic Priorities



5.3 Results framework

Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
1. Expand Multisectoral Collaboration to Address NCDs & Mental Health Across Life Course	1.1. NCD and risk factors management and control	- Capacity Building (HRH and institutions) - Facilitate intersectoral collaboration for behavioral risk reduction - Support Evidence based RCCE for NCD prevention and management	1.1 Support the Ministry of Health, Bhutan in the prevention, promotion and management of NCDs including expanding efforts to address risk factors. 1.2 Support the development and implementation of the national multisectoral action plan for NCD prevention and control (MSAPII). 1.3 Expand implementation of WHO NCD Best Buys tailored across various life stages.	- All partners maintain their commitment to the MSAP II - Adequate and sustained financing - Partners have adequate capacities to implement MSAP II - Community engagement in health promotion.	- Increased access to quality NCD services at PHC level - Improved healthy behaviors through targeted interventions - Strengthened policy and regulatory environment for NCD prevention - Community engagement in health promotion.	Reduced deaths due to NCDs by one third from 2015 (MSAP II)
	1.2. Cancer	- Support HPV vaccination and screening efforts - Promote advocacy efforts towards risk factor reduction - Facilitate cross-sector collaboration - Capacity Building support to healthcare providers for early detection and diagnosis	2.1 Sustain and strengthen HPV vaccination coverage and integrate cervical cancer screening into primary healthcare 2.2 Support the scale up cervical, gastric, and breast cancer screening programmes, with enhanced access to HPV DNA testing, endoscopy services, and mammography including underserved populations.	- Sustained political commitment to cancer prevention and screening - Adequate funding for vaccination, screening programs, and diagnostic capacity expansion	- Trained healthcare workforce in cancer detection, diagnosis, and treatment - Engagement of education and community sectors for cancer awareness campaigns - Strong collaboration with food safety, tobacco control, and nutrition sectors	

Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
(CCS 2026–2030)	1.3. Mental Health	- Facilitate Multisectoral Collaboration to address Mental Health across the life course - Capacity Building of healthcare providers in early detection, diagnosis, and management of mental health conditions - Advocate for policy frameworks to strengthen mental health governance	3.1 Support the Mental Health Gap Action Programme (MhGAP) and LIVE LIFE and any other technical packages for suicide prevention.	- Government commitment to mental health policy and funding - Community engagement and support from schools, workplaces, and social services	<i>Results that WHO contributes to</i> - Integration of mental health in PHC with clear referral pathways - Adequate human resources trained in mental health services and psychosocial support	
Prevention and Control of Communicable Disease	1. Malaria Prevention & Control including other NTDs	- Enhance prevention, detection, and treatment of malaria and other NTDs - Strengthen disease surveillance, vector control, and cross-sectoral collaboration	1.1 Sustain and strengthen Bhutan's malaria elimination efforts and control programs for neglected tropical diseases such as kala-azar, leprosy, and rabies to achieve and maintain elimination.	- Sustained government commitment and regional collaboration - Effective Cross-border collaboration	- Malaria and priority NTDs such as Kala-azar, Leprosy & Rabies remain eliminated - Strong community engagement and public health awareness - Improved health worker capacity for NTD surveillance and treatment	Reduced incidence on communicable diseases

Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
(CCS 2026–2030)		<i>Actions that WHO will take</i>	<i>Results that WHO will achieve</i>	<i>What WHO needs to achieve the results</i>	<i>Results that WHO contributes to</i>	
	2. HIV/STI	1. Policy and Guideline development 2. Public Health Advocacy support	2.1 Support and strengthen the National STI and HIV/AIDS Control Programme	1. Government commitment to HIV/STI prevention policies 2. Sustained funding and resources for HIV/STI programs 3. Community engagement to address stigma and promote early testing 4. Collaboration with education, social services, and civil society organizations	1. Strong program and institution 2. Capacitated health workforce 3. Improved access and performance of HIV//STI services	
	3. Tuberculosis	1. Strengthen TB prevention, detection, and treatment through guidelines, evidence, and strategic interventions. 2. Enhance TB surveillance and reporting with policy, technical support, and convening efforts.	3.1 Support and strengthen the National Tuberculosis Control Programme	1. Sustained political commitment and financing for TB control programs 2. Community and civil society engagement to ensure equitable access	1. Health system capacity for early detection, treatment, and follow-up strengthened 2. Stronger Cross-border collaboration to prevent TB transmission¹.	

Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
(CCS 2026–2030)	4. Infectious Diseases Prevention and control	1. Strengthen surveillance systems for early detection, reporting, and response to infectious diseases 2. Enhance outbreak preparedness and response mechanisms	4.1 Strengthen systems for infectious disease prevention and control including supporting the implementation of the National Action Plan on AMR (2024–2028).	1. Sustained government commitment and policy alignment with global health security frameworks. 2. Adequate financing and resources for surveillance, preparedness, and response. 3. Training and capacity building for healthcare workers in infectious disease control. 4. Collaboration with regional and global health organizations to strengthen disease monitoring and response systems.	Results that WHO contributes to 1. Bhutan is well-prepared for any outbreaks	

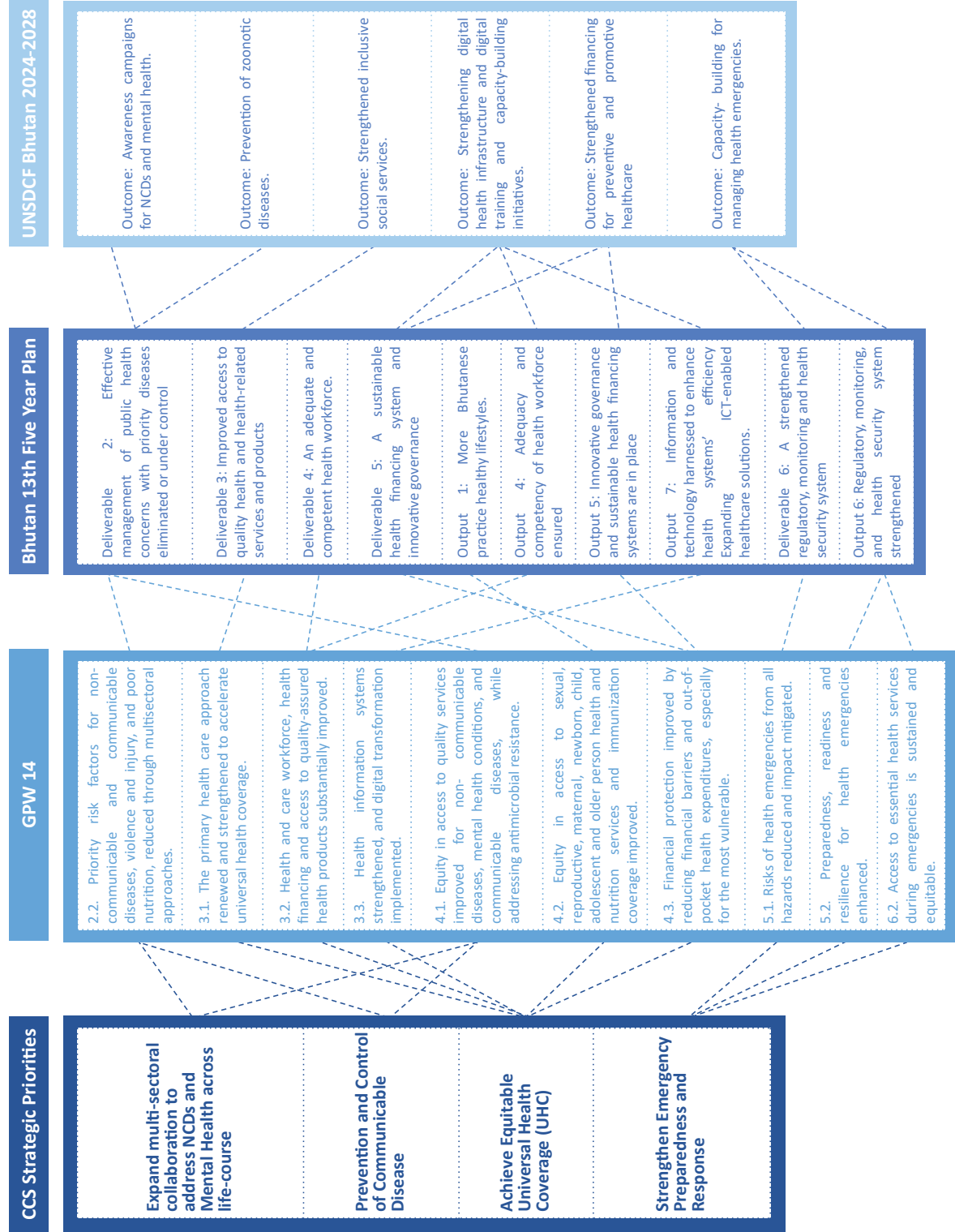
Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
(CCS 2026–2030)		<i>Actions that WHO will take</i>	<i>Results that WHO will achieve</i>	<i>What WHO needs to achieve the results</i>	<i>Results that WHO contributes to</i>	
Achieve Equitable Universal Health Coverage (UHC)	1. Comprehensive Primary Health Care & Immunization	1. Expand telemedicine and mobile health units. 2. Roll out the PEN package nationally. 3. Conduct participatory community health planning.	1.1. Support and strengthen the SRMNCAH Strategy 2024–2029 using a life-course approach. 1.2 Strengthen the National Immunization Strategy (NIS) 2024–2028 1.3 Review and strengthen PHC-orientation of health systems 1.4 Achieve maturity level 2 for medicines and vaccines regulatory system and maturity level 3 in priority domains against the global benchmarking tool.	1. Robust health policies aligning with UHC. 2. Enhanced resource allocation for PHC. 3. Stakeholder collaboration for RMNCAH implementation.	1. All eligible individuals are vaccinated per plan	People receiving essential health services without suffering financial hardship
	2. Digital Health and Telemedicine	4. 1. Support expansion of digital public infrastructure and e-health platforms to improve health information systems. 5. 2. Enhance telemedicine services to bridge healthcare access gaps in rural and remote areas.	2.1 Support the implementation of national digital health strategy including e-PIS and other digital health platforms	1. Supportive regulatory frameworks for telemedicine. 2. Strengthened digital health policies and data governance.	1. Reduction in preventable hospitalizations and health complications due to timely access to telemedicine services 2. Reduction of Out-Of-Pocket Expenditure	

Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
(CCS 2026–2030)						
	3. Human Resources for Health	1. Strengthen workforce planning, training, and retention of healthcare professionals. 2. Improve deployment strategies to ensure equitable distribution of health workers, especially in rural areas.	3.1 Support the development and implementation of Bhutan's HRH strategy & strengthen HRIS to address workforce gaps & retention issues 3.2 Sustain capacity-building programmes and mentorship initiatives for HRH on clinical and non-clinical priority topics as well as strengthening regulatory capacity of the Bhutan Medical and Health Professions Council	1. Implementation of Bhutan's HRH strategy. 2. Strong partnerships with academic and training institutions.	1. Adequate, relevant, competent health workforce 2. Strong pipeline of health workforce	
	4. Sustainable Health Financing	1. Technical guidance	4.1 Support the Ministry of Health towards sustainable & equitable health financing strategy	1. Sustainable and equitable health financing system reducing financial barriers to care. 2. Government commitment to health financing reforms. 3. Private sector governance and partnerships for resource mobilization. 4. Efficient budget allocation and expenditure management in health.	1. Reduction in out-of-pocket health expenditures as a percentage of total health spending 2. Increase in domestic spending as a percentage of GDP 3. Increased proportion of essential health services funded through predictable and sustainable sources	

Strategic Priority	Focus Area	Strategic Actions	Deliverables	Enabling Environment	Strategic Outcome	Indicators
Strengthen Emergency Preparedness & Response (CCS 2026–2030)	1. Health Emergency Prevention & Preparedness	1. Technical Guidance 2. Institutional capacity building support	1.1 Develop and implement guidelines and SOPs to ensure continuity and equitable access to essential health services during large-scale emergencies. 1.2 Support NAPHS development and implementation based on JEE and IHR review findings, with emphasis on institutional capacity strengthening for emergency preparedness.	1. Integration of IHR into health policies.	1. Strengthen public health preparedness and response 2. Clear legal frameworks to detect, report, and respond to public health threats	Reduced time taken to detect, report, and respond to a public health emergency in compliance with International Health Regulations (IHR)
	2. Zoonotic and emerging diseases prevention and preparedness	1. Support in strengthening surveillance systems. 2. Multi-sectoral collaboration and technical guidance	2.1 Institutionalize multi-source collaborative surveillance systems integrating data from different sectors and sources 2.2 Fostering inter-sectoral collaboration and capacity enhancement to strengthen preparedness for zoonotic and emerging diseases under the One Health framework.	1. Strengthened multi-sectoral emergency preparedness mechanisms.		

Refer to section 8.6 for list of targets

5.4 Alignment of WHO Bhutan Strategic Priorities 2026–2030 with GPW 14, UNSDCF, 13th Five-Year Plan



Refer to Table 2 in Appendices for more details.

6. Implementation of CCS

The implementation of CCS 2026–2030 will leverage the strengths of the WHO support across the Organization’s three levels – Country Office, Regional Office and headquarters – to ensure the effective realization of the Strategic Priorities. The CCS will focus on strengthening national systems, intersectoral collaboration and strategic partnerships to achieve the outlined priorities and deliver tangible health outcomes. **Recognizing that health outcomes are influenced by broader social, economic and environmental determinants, the CCS underscores the importance of cross-sectoral collaboration and multistakeholder partnerships to drive sustainable progress.** The following sub-sections detail the implementation mechanisms, roles of stakeholders and partnerships necessary for success.

6.1 Collaborative mechanisms: Define the roles of WHO, national authorities and other stakeholders

Country Office	Regional Office	Headquarters
Strengthen in-country presence: Maintain a skilled multidisciplinary team working closely with the Royal Government of Bhutan to align biennial work plans with national health priorities.	Technical coordination and support: Provide backstopping for technical assistance, simulation exercises and capacity-building programmes, including disease surveillance and climate-resilient health systems.	Global standards and guidelines: Develop evidence-based policies, methodologies and technical guidelines tailored to the evolving health challenges of Bhutan.
Policy and technical guidance: Collaborate with the Ministry of Health (MoH), the Ministry of Finance and other stakeholders to develop policies and strategies for priority health programmes, including UHC and NCD prevention. Provide technical support to design and implement innovative health financing models.	South–South and triangular cooperation: Facilitate knowledge-sharing and collaboration between Bhutan and other countries in the Region to replicate successful health interventions and innovations.	International alignment: Facilitate Bhutan’s integration into global health initiatives, ensuring alignment with WHO GPW 14, SDGs and other international frameworks.
Resource mobilization: Facilitate funding for health programmes by engaging development partners, UN agencies and NGOs to address health inequities and gaps in service delivery.	Monitoring and evaluation: Assist in the development and execution of robust monitoring frameworks, including reviews of IHR (2005) Core Capacities and periodic evaluations of health outcomes.	Documentation and knowledge-sharing: Document Bhutan’s health achievements and share success stories globally, showcasing best practices in areas such as cervical cancer prevention and emergency preparedness.
Community engagement: Strengthen mechanisms for community-level participation in health planning and service delivery, particularly in underserved areas.	Regional advocacy and networking: Advocate for regional collaboration and mobilize funds to strengthen Bhutan’s alignment with regional health priorities.	Research and innovation: Support the adaptation of global tools to Bhutan’s context, focusing on digital health innovations, climate-adaptive health systems, and community-level interventions.

Country Office	Regional Office	Headquarters
Capacity-building: Support national and district health teams through tailored training programmes in emergency preparedness, disease surveillance and digital health implementation.		

6.2 Multisectoral approaches

- **Social determinants of health:** Foster partnerships between health and non-health sectors to address critical determinants, such as education, water and sanitation (WASH), gender equality and digital health innovation. Furthermore, regulating commercial influences by curbing unhealthy product marketing and raising consumer awareness can help mitigate health risks and support public health priorities.
- **Climate-health nexus:** Collaborate with the Royal Government of Bhutan and local governments to implement action plans for climate change adaptation, focusing on building climate-resilient health systems and promoting low-carbon development at health-care facilities.
- **Community engagement:** Strengthen mechanisms for community engagement in health planning and service delivery, particularly in rural and underserved areas, ensuring inclusive and people-centred care.
- **Action plans:** Establish intersectoral platforms for creating and executing action plans that integrate Health in All Policies (HiAP), with an emphasis on NCD prevention, antimicrobial resistance (AMR), health promotion and environmental health.
- **Capacity-building:** Equip stakeholders across sectors with the tools and skills required for implementing and monitoring multisectoral approaches effectively.

6.3 Partnerships with UN and other other agencies

- **UN agencies:** Align CCS implementation with UNSDCF Bhutan (2024–2028) by fostering partnerships with key UN agencies such as UNICEF, UNFPA, UNDP and WFP, focusing on shared priorities such as RMNCAH, immunization, health equity and emergency preparedness.
- **NGO and civil society engagement:** Work with NGOs, CSOs and community-based organizations (CBOs).
- **Academic and research institutions:** Partner with institutions such as Khesar Gyalpo University of Medical Sciences of Bhutan (KGUMSB) and PEMA to strengthen local research, innovation and workforce development capabilities.
- **Multilateral development banks:** MDBs, especially World Bank and Asian Development Bank, make significant contributions to Bhutan’s health systems strengthening response. WHO’s involvement would improve prioritization, optimization and effectiveness of the investments.

- **Bilateral agencies:** Bilateral agencies, such as JICA, Korea International Cooperation Agency and Austrian Development Agency, make important contributions to health systems and WHO's partnership will support aid effectiveness.
- **Private sector involvement:** WHO will support the government in developing its private sector engagement strategy, as it emerges.
- **Intergovernmental cooperation:** Bhutan enjoys partnerships with Bangladesh, India, Japan, Singapore and Thailand on medical equipment, specialist sharing, capacity-building, hospital management, etc. WHO's role would enhance optimization through support to the government.
- **The Royal Government of Bhutan leadership:** Collaborate with the Royal Government of Bhutan to ensure cohesive implementation of health initiatives, leveraging its leadership in integrating health into national development policies.

Success will look like:

Strengthened leadership and governance: enhanced governance structures and stronger interministerial coordination supporting universal health coverage (UHC) and resilience

Improved service delivery: increased access to equitable, high-quality health services, especially in underserved regions and across all life stages

Health equity achievements: significant reduction in health disparities for people living in vulnerable conditions or circumstances, including rural populations, women and children

Innovative resource mobilization: sustainable health financing through innovative funding mechanisms, including public–private partnerships and donor engagement

Enhanced system resilience: robust emergency preparedness and response systems capable of addressing both existing and emerging health threats, with a focus on climate change adaptation

7. Monitoring and evaluation

The monitoring and evaluation (M&E) framework for the CCS 2026–2030 in Bhutan is designed to ensure effective tracking of progress, timely identification of challenges, and necessary adjustments to improve health outcomes and indicators. The M&E framework aligns with the WHO Fourteenth General Programme of Work (GPW 14), Bhutan’s 13th Five-Year Plan (FYP) and the United Nations Sustainable Development Cooperation Framework (UNSDCF) 2024–2028.

7.1 Monitoring CCS implementation

Progress in the implementation of the CCS 2026–2030 will be assessed using a proposed three-tier approach: **annual monitoring, mid-term review and final evaluation.**

Annual monitoring (2026–2030)

Annual monitoring will be conducted by the WHO Country Office in collaboration with the Ministry of Health (MoH) to regularly track the progress of CCS implementation. This process will involve annual review meetings assessing key indicators and progress on Strategic Priorities. The process would also ensure that the CCS priorities align with Bhutan’s health policies and are reflected in biennial work plans. Wherever necessary, WHO will provide support to build M&E competencies among the Country Office staff to enhance results-based monitoring.

Mid-term review (2028)

A mid-term review will be conducted in 2028 to assess progress, identify bottlenecks and recommend course corrections. The review will be evidence-based and objective, assessing the effectiveness of interventions and recommending mid-course adjustments for improved implementation.

The best practices and lessons learnt will be documented to refine strategies.

Final evaluation (2030)

At the end of the CCS cycle in 2030, a comprehensive evaluation will be conducted to assess progress against CCS indicators and Bhutan’s national health goals.

A comprehensive evaluation will primarily examine Bhutan’s contributions to regional and global health objectives, examine the extent to which Bhutan has institutionalized and sustained WHO-supported programmes, and identify policy recommendations for future CCS cycles and WHO’s engagement in Bhutan.

In addition to the monitoring mechanism, Bhutan’s CCS implementation will also integrate an adaptive learning approach, ensuring continuous monitoring to enable real-time adjustments for data-driven changes, regular dialogues with stakeholders and development partners for coordinated efforts, and documentation and knowledge-sharing of best practices and lessons learnt disseminated through reports and workshops.

7.2 Monitoring and evaluation framework

The M&E framework defines key indicators to track progress towards strategic outcomes and targets, ensuring that Bhutan's CCS remains adaptive and effective. The identified indicators should be reported annually as part of the review to inform the annual/biennial planning process. Suggested indicators are in Appendices Section 8.6, which will need to be aligned once the biennium plans are ready.

8. Appendices

8.1 Key health indicators (in continuation)

(a) Maternal and child health

Maternal and child health outcomes in Bhutan have seen marked improvement over the years. Maternal mortality has dropped dramatically from 777 deaths per 100 000 live births in 1984 to 53 in 2023, highlighting the country's focus on expanding access to skilled birth attendance and institutional deliveries. Similarly, infant and under-five mortality rates have seen a steady decline, supported by improved immunization coverage, better antenatal care and targeted interventions for neonatal health. While these achievements are significant, challenges persist, particularly in addressing anaemia among pregnant women and adolescents. According to National Health Survey 2023, anaemia affects 33.3% of pregnant women, posing risks to maternal and neonatal health, including low birth weight, preterm delivery and maternal complications. Among adolescents (aged 10–19), anaemia prevalence remains high at 36.5%, highlighting broader nutritional challenges and the need for targeted interventions.

Disparities between urban and rural areas persist in accessing maternal and child health services. Bhutan's rugged terrain and limited infrastructure in remote areas create barriers to care, making it challenging to provide equitable health services. Community-based health programmes, outreach services and strengthened capacity at basic health units (BHUs) are critical to overcoming these barriers and ensuring equitable health care delivery.

Efforts to address adolescent health, including anaemia, reproductive health and mental well-being, are pivotal in shaping a healthier future generation. Similarly, combatting anaemia among pregnant women requires integrated interventions such as nutrition education, routine screening during antenatal care and improved access to supplementation programmes. These initiatives align with Bhutan's 13th Five-Year Plan³⁹ objectives to enhance maternal and child health, promote nutritional security and reduce disparities in health care access. By addressing these challenges, Bhutan can further consolidate its progress and move closer to achieving universal health coverage and Sustainable Development Goals (SDGs).

(b) Fertility trends

Bhutan's total fertility rate (TFR) has seen a substantial decline, dropping from 5.6 births per woman in 1994 to approximately 2 in 2023. This reduction indicates successful implementation of family planning initiatives and increased access to reproductive health services. However, the general fertility rate (GFR), which measures the number of births per 1000 women aged 15–49 years, has shown a slight increase from 72 in 2012 to 75.2 in 2023. This uptick may suggest regional or demographic

³⁹ Bhutan's 13th Five-Year Plan (FYP) prioritizes health as a key pillar of socioeconomic development, aligning with the nation's gross national happiness (GNH) philosophy and the Sustainable Development Goals (SDGs). The plan emphasizes strengthening primary health care (PHC) in rural and remote areas, addressing the growing burden of noncommunicable diseases (NCDs) through prevention, early detection and integration into PHC, and further reducing maternal and child mortality through improved access to skilled birth attendance and immunization programmes. Climate adaptation is integrated into health systems to mitigate risks from climate change, while investments in training and international partnerships aim to address critical human resource shortages.

disparities that warrant further investigation to ensure equitable access to family planning resources across all segments of the population.

(c) Disease burden and trends

Bhutan faces a dual burden of communicable and noncommunicable diseases. While the country has made significant progress in addressing communicable diseases such as tuberculosis, malaria, rabies, kala-azar and leprosy, the growing prevalence of NCDs presents a new set of challenges. ***Noncommunicable diseases, including cardiovascular diseases, diabetes and cancers, now account for nearly 70% of Bhutan's disease burden.***⁴⁰ Risk factors such as tobacco use, physical inactivity and unhealthy diets are on the rise. Tobacco use increased from 23.9% in 2019 to 31.4 % in 2023, and the prevalence of high blood pressure has climbed to 30.1%. The 2019 STEPS Survey,⁴¹ conducted by Bhutan's Ministry of Health in collaboration with the World Health Organization (WHO) and National Health Survey (NHS) 2023, offers critical insights into these risk factors and their implications for health policy.

NHS 2023 revealed that 31.4% of Bhutanese people are current users of tobacco with men (42.7%) more likely to use any tobacco than women (18.5%). Smokeless tobacco, such as chewing tobacco, is particularly prevalent at 21.1%. Additionally, 18.2% of individuals are exposed to second-hand smoke at home, emphasizing the need for stricter enforcement of tobacco control policies to protect public health. Alcohol consumption remains another significant concern, with 40.2% of respondents identified as current drinkers and 13.3% engaged in heavy episodic drinking. Cultural practices and availability of home-brewed alcohol contribute to these patterns, which pose risks to both individual and public health.

Bhutan's life expectancy at birth has increased significantly over the past two decades, from 66 years in 2000 to 74.9 years in 2021. Similarly, healthy life expectancy (HALE) has risen from 57.2 years to 64.8 years, indicating notable progress in reducing mortality from lower respiratory infections and diarrhoeal diseases.⁴² However, the gap between life expectancy and HALE has widened, increasing from 8.6 years in 2000 to 9.7 years in 2021, due to an increasing burden of chronic conditions such as ischaemic heart disease, chronic obstructive pulmonary disease (COPD), stroke and tuberculosis. These trends underscore the need for an integrated approach to managing NCDs and addressing risk factors to improve the quality of life, especially in later years.

Cancers constitute a significant proportion of Bhutan's NCD burden. The Bhutan Cancer Control Strategy 2019–2025 reports that between 2014 and 2017, there were 2648 recorded cancer cases in Bhutan, with 1156 cases among men and 1492 among women. The top five cancers during this period were cervical, stomach, head and neck, lung, and oesophageal cancers. The high prevalence of stomach cancer is linked to *Helicobacter pylori* infections, with 66%–82% of Bhutanese people found to be carrying the bacteria. Bhutan's Cancer Control Strategy (2019–2025) and related screening guidelines developed for gastric, cervical and breast cancers integrate evidence-based approaches with culturally appropriate methodologies. Bhutan's use of visual inspection with acetic acid (VIA) and HPV vaccination demonstrates its leadership in regional cervical cancer prevention.⁴³

40 <https://www.moh.gov.bt/wp-content/uploads/ict-files/2021/06/BUEPRINT-HEALTH-FLAGSHIP-PROJECT.pdf>

41 <https://cdn.who.int/media/docs/default-source/searo/ncd/noncommunicable-disease-risk-factors-bhutan-steps-survey-report-2019.pdf>

42 Bhutan

43 Evaluation of adoption of people-centred NCD service delivery within primary health care in WHO South-East Asia Region 2014–2021. New Delhi: World Health Organization, Regional Office for South-East Asia; 2024. Licence: CC BY-NC-SA

Cervical cancer: Cervical cancer remains the leading cancer among women in Bhutan, particularly affecting those between 15 and 44 years of age. Bhutan has implemented a National Cervical Cancer Screening Programme targeting women aged 25–65 years. Screening is integrated into the routine health-care system through primary health centres and district hospitals. The primary methods used are cytology (Pap smears) and visual inspection with acetic acid (VIA). VIA, in particular, has proven to be a cost-effective alternative in resource-constrained settings, offering immediate treatment opportunities for precancerous lesions (ICO/IARC, 2023).⁴⁴ In 2022, Bhutan introduced the HPV DNA test, a more specific and precise test.

Despite these efforts, significant gaps remain. As of 2019, an estimated 37% of eligible women aged 30–49 years had been screened within the last five years while only 29% had been screened within the last three years. These figures highlight the need for expanded outreach to rural and remote populations, where access to health-care services is limited due to geographical barriers (ICO/IARC, 2023). Strengthening active invitation systems and community engagement may help improve screening rates and address disparities.

Additionally, Bhutan’s cervical cancer prevention efforts include a highly successful HPV Vaccination Programme. Initiated in 2010, this programme achieved coverage rates of 89% for the first dose and 88% for the last dose among eligible women by 2021 (ICO/IARC, 2023). These initiatives, combined with ongoing efforts to enhance screening coverage, position Bhutan as a leader in cervical cancer prevention in the Region. However, further efforts are required to ensure equitable access and sustained progress.

HIV/AIDS: Bhutan has maintained a low prevalence of HIV. Yet the country faces challenges related to late diagnoses and stigma associated with the disease. According to the National HIV/AIDS Control Programme, in 2022, Bhutan reported 79 new HIV cases, including 42 men and 37 women, with five cases among young adults aged 15–24 years.⁴⁵ This highlights the need for increased awareness campaigns, better access to testing and improved confidentiality in service delivery.

Tuberculosis: TB remains a public health challenge in Bhutan, with an incidence rate of approximately 126 cases per 100 000 population in 2022, only slightly lower than the global incidence rate of 134 (2023).⁴⁶ Although the country has made progress in TB detection and treatment, multidrug-resistant TB (MDR-TB) presents growing concerns. Notification rates for TB remain consistent, and Bhutan adheres to the directly observed treatment, short-course (DOTS) strategy for treatment compliance. Active case-finding strategies and public health awareness programmes have contributed to high treatment success rates.

Malaria: Bhutan has remarkably reduced malaria cases, transitioning towards elimination. The incidence of malaria dropped from over 39 000 cases in the 1990s to fewer than five indigenous cases in 2022, which is significantly lower than the global incidence of malaria estimated at 58.9.⁴⁷ The Vector-Borne Disease Control Programme (VDCP) has been pivotal in achieving this reduction. Bhutan is on track to achieve malaria-free status by 2025, supported by robust surveillance systems and vector control measures. In addition to the “big three” (malaria, HIV/AIDS and tuberculosis), Bhutan contends with other communicable diseases such as hepatitis, diarrhoeal diseases and respiratory

3.0 IGO.

44 https://hpvcentre.net/statistics/reports/BTN_FS.pdf

45 <https://www.moh.gov.bt/wp-content/uploads/Annual-Health-Bulleti-2023.pdf>

46 <https://data.worldbank.org/indicator/SH.TBS.INCD>

47 <https://data.worldbank.org/indicator/SH.MLR.INCD.P3>

infections. For example, hepatitis B – acute morbidity rates stand at 9.6 cases per 100 000 population ; waterborne diseases – diarrhoeal diseases remain a concern, particularly among children under five, despite improved sanitation coverage reaching 94.6% of the population.

Bhutan has also made progress in the elimination of priority neglected tropical diseases. Kala-azar cases are now sporadic, occurring only every few years. There have been zero reported human deaths from rabies since 2024. Leprosy incidence has drastically declined, with fewer than 20 new cases in the last decade and fewer than 10 new cases since 2022. These achievements reflect the country's commitment to public health and its success in implementing integrated disease surveillance and response strategies.

(d) Routine immunization coverage

NHS 2023 reveals that Bhutan maintains high routine immunization coverage, with over 99% of eligible children receiving critical vaccinations such as Bacillus Calmette-Guérin (BCG), oral polio vaccine (OPV) and measles-rubella (MR) vaccines. Such widespread immunization coverage has played a pivotal role in the elimination of diseases such as maternal and neonatal tetanus and measles in Bhutan.

(e) Vaccination coverage

Bhutan has emerged as a global leader in HPV vaccination, demonstrating remarkable success in its efforts to prevent cervical cancer. In 2010, Bhutan launched a nationwide HPV Vaccination Programme, targeting girls aged 12–18 years with vaccination against four of the most common and carcinogenic HPV types. ***The campaign achieved an initial coverage of approximately 90% of the target age group, a testament to the country's robust health-care infrastructure and strong public health commitment.***⁴⁸ Since 2014, the programme has transitioned to a school-based delivery model, maintaining high vaccination rates and ensuring sustained coverage. In 2020, Bhutan also pioneered a school-based gender-neutral human papillomavirus (HPV) vaccination programme, achieving an impressive 96% vaccination coverage rate by 2021.⁴⁹

Research conducted by the International Agency for Research on Cancer (IARC) and Bhutan's Ministry of Health underscores the programme's significant impact. Comparing data from 2011–2012 with 2018, the prevalence of vaccine-targeted HPV types among women aged 17–29 years in Thimphu, the capital of Bhutan, decreased by an impressive 93%. Unvaccinated women also benefited from reduced community transmission, with a prevalence reduction of 88% for vaccine-targeted HPV types. These findings demonstrate the programme's effectiveness not only in direct protection but also in achieving herd immunity.

The COVID-19 pandemic tested Bhutan's health system resilience and the country rose to the challenge with an exceptional vaccination campaign. Bhutan achieved **one of the fastest mass vaccinations globally**, with over **90% of the eligible population** fully vaccinated within weeks of receiving vaccine supplies.⁵⁰ This success was made possible through:⁵¹

48 <https://www.iarc.who.int/news-events/towards-cervical-cancer-elimination-through-hpv-vaccination-in-bhutan/>

49 <https://pubmed.ncbi.nlm.nih.gov/39169479/#:~:text=The%20study%20concludes%20that%20extending,gender%2Dneutral%20HPV%20vaccination%20program.>

50 <https://www.unicef.org/bulgaria/en/stories/unicef-hails-bhutans-successful-completion-full-covid-19-vaccinations-90-population#:~:text=THIMPHU%2C%2027%20JULY%202021%20%2D%20E2%80%9C,the%20country%20has%20been%20vaccinated.>

51 <https://idpjournal.biomedcentral.com/articles/10.1186/s40249-021-00929-x#:~:text=Enabling%20factors%20of%20>

- **International partnerships:** Bhutan leveraged its diplomatic relationships to secure vaccine donations and procurements from countries such as India and international agencies.
- **Community engagement:** Health workers, village volunteers and local leaders ensured timely vaccine delivery even in the most remote regions.
- **Logistics planning:** Bhutan's Ministry of Health orchestrated a well-coordinated campaign, setting up temporary vaccination centres and ensuring cold chain maintenance in mountainous terrains.

(f) Mental health: Data submitted by health facilities in the country show increasing trends in the number of mental health cases

The burden of mental health conditions					
Type of disorders	2016	2017	2018	2019	2020
Dementia	21	18	23	28	36
Alcohol use disorder	1024	925	1011	1346	1505
Substance use disorder	282	193	326	326	303
Psychosis	318	160	239	391	463
Depression	743	503	702	762	1190
Anxiety	2141	1147	1318	1370	2164
Epilepsy	1537	1132	1122	1125	1491
Others	1724	1354	1438	1730	1233

Source: Annual Health Bulletin 2020, Ministry of Health, Royal Government of Bhutan

8.2 Environmental and climate change impact on health

Bhutan's unique mountainous geography and growing vulnerability to climate change present profound risks to public health, necessitating urgent and coordinated responses. The country's rugged terrain makes many communities inherently isolated while climate-related hazards exacerbate existing challenges in accessing and delivering health care. Bhutan is particularly susceptible to glacial lake outburst floods (GLOFs), landslides, flash floods and extreme weather events, all of which threaten lives, destroy infrastructure and disrupt health-care services, especially in remote and underserved areas (UNSDPF 2024–2028).

Bhutan's vulnerable ecosystems also contribute to the emergence and spread of climate-sensitive diseases. Vector-borne diseases such as malaria, dengue fever and kala-azar, which had been largely under control, are now re-emerging in regions previously considered unaffected due to changing temperature and precipitation patterns (WHO Bhutan Climate Report, 2023) (UNDP Report, 2023). In light of these multifaceted challenges, *Bhutan's National Adaptation Plan* (NAP) emphasizes the need for climate-resilient health-care systems, early warning systems and sustainable infrastructure development to mitigate the health impact of climate change.

this%20successful,figures%2C%20volunteers%20and%20local%20leaders.

Strengthening collaboration with regional and global partners, including leveraging technology for telemedicine and climate monitoring, is critical to ensure that health-care services remain accessible to all, particularly in hard-to-reach areas.

8.3 Digital health and innovation

Bhutan's progress in digital health offers an opportunity to bridge access gaps, particularly in remote areas. Initiatives such as telemedicine and e-health services have enhanced health care delivery. However, barriers such as an insufficient pool of skilled IT professionals, limited digital literacy, infrastructure constraints and data privacy concerns remain. Strengthening digital health policies and expanding internet connectivity are critical for scaling these innovations.

8.4 WHO's collaborative history with Bhutan

Year	Milestone
↓ 1983	Establishment of the WHO Country Office in Bhutan
↓ 1988	Launch of Bhutan's Expanded Programme on Immunization (EPI) with WHO support, significantly reducing vaccine-preventable diseases
↓ 1997	Bhutan declared polio-free, with WHO playing a critical role in immunization campaigns and surveillance efforts
↓ 2002	Development of Bhutan's first CCS 2002–2007, focusing on strengthening primary health care, immunization and capacity-building for health system management
↓ 2004	WHO assisted Bhutan in implementing the Framework Convention on Tobacco Control, making Bhutan the first country to ban tobacco sales.
↓ 2009	Development of second CCS 2009–2013 focusing on strengthening health systems, addressing MDGs and enhancing health partnerships
↓ 2014	Bhutan received Polio-Free Certification from WHO.
↓ 2014	The third CCS (2014–2018) focused on achieving UHC, reducing health inequities, addressing emerging health challenges such as NCDs and mental health, and strengthening emergency preparedness and response.
↓ 2016	Maternal and neonatal tetanus elimination achieved
↓ 2017	Measles elimination certified and elimination sustained
↓ 2017	Bhutan conferred the World No Tobacco Day Award for exemplary efforts in tobacco control
↓ 2017	National Action Plan on Antimicrobial Resistance (AMR) 2018–2022 launched in November
↓ 2018	Rubella and Congenital Rubella Syndrome brought under control
↓ 2020	Bhutan made significant strides in health system resilience during the COVID-19 pandemic with WHO guidance.
↓ 2020	Development of CCS 2020–2024, focusing on UHC, equity and health systems strengthening
↓ 2023	Achieved the elimination of rubella, meeting the regional target of eliminating measles and rubella

Table 2: Alignment of WHO Bhutan Strategic Priorities 2026–2030, alignment with GPW 14, UNSDCF, 13th Five Year Plan and SDGs

WHO Strategic Priorities 2026–2030	GPW 14 alignment	Bhutan 13th Five-Year Plan	UNSDCF Bhutan 2024–2028 alignment	SDG alignment	GPW 14 output prioritization
Expand multisectoral collaboration to address NCDs and mental health across life-course.	2.2: Risk factors for NCDs and mental health reduced 4.1: Equity in NCD and mental health services improved	Deliverable 2: Effective management of public health concerns with priority diseases eliminated or under control Addressing NCD risk factors and mental health	Outcome 2: Awareness campaigns for NCDs and mental health	SDG 3, SDG 11, SDG 13.	Outcome: 2.2 and 4.1 Output: 2.2.1, 2.2.2, 4.1.1 and 4.1.2 Priority level: HIGH
Prevention and control of communicable diseases	2.2: Risk factors for communicable diseases reduced 4.1: Equity in communicable diseases services while addressing antimicrobial resistance	Deliverable 2: Effective management of public health concerns with priority diseases eliminated or under control	Outcome: Prevention of zoonotic diseases	SDG 3	Outcome: 2.2 and 4.1 Output: 2.2.1 and 4.1.3 Priority level: HIGH
Achieve equitable universal health coverage (UHC).	3.1. The primary health care approach renewed and strengthened to accelerate universal health coverage 3.2. Health and care workforce, health financing and access to quality-assured health products substantially improved 3.3. Health information systems strengthened and digital transformation implemented 4.2. Equity in access to SRVIMNACH and immunization coverage improved 4.3 Financial protection improved	Outcome 1: By 2029, more Bhutanese people enjoy improved health and well-being. Output 1: More Bhutanese people practise healthy lifestyles. Deliverable 3: Improved access to quality health and health-related services and products Strategy 2: Investing in cost-effective preventive and promotive health care Output 7: Information and technology harnessed to enhance health systems' efficiency, expanding ICT-enabled health-care solutions.	Outcome 2: Strengthened inclusive social services Outcome 2: Strengthening digital health infrastructure and digital training and capacity-building initiatives Outcome 2: Strengthened financing for preventive and promotive health care	SDG 3 (good health), SDG 10 (reduced inequalities) SDG 9 (innovation), SDG 3, SDG 10. SDG 17, SDG 13	Outcome: 3.1, 3.2, 4.2 and 4.3 Output: 3.1.1, 3.1.2, 3.2.1, 3.3.1, 4.2.1, 4.2.2, Priority level: HIGH

WHO Strategic Priorities 2026–2030	GPW 14 alignment	Bhutan 13th Five-Year Plan	UNSDCF Bhutan 2024–2028 alignment	SDG alignment	GPW 14 output prioritization
		Output 4: Adequacy and competency of health workforce ensured Deliverable 4: An adequate and competent health workforce Strategy 1: Improving the health governance system and health-care delivery services Output 5: Innovative governance and sustainable health financing systems in place Deliverable 5: Sustainable health financing system; reduced out-of-pocket expenses			
Strengthen emergency preparedness and response	5.1: Risks of emergencies reduced 5.2: Preparedness for emergencies enhanced 6.2: Access to essential health services during emergencies is sustained and equitable.	Deliverable 6: A strengthened regulatory, monitoring and health security system Output 6: Regulatory, monitoring and health security system strengthened Disaster resilience through better risk management	Outcome 2: Capacity-building for managing health emergencies	SDG 3, SDG 13, SDG 11 (sustainable cities)	Outcome: 5.2 Output: 5.2.1 Priority level: HIGH

8.5 Stakeholders interviewed for CCS development

	Date	Organization	Name	Position
1	20 Jan. 2025	WHO-SEARO	Dr Misbah Sheikh	Head, Partnerships and Resource Mobilization
2	23 Jan. 2025	WHO HQ	Dr Indrajit Hazarika	Country Strategy and Support
3	24 Jan. 2025	WHO-SEARO	Dr Kishori Mahat	Country Support Unit
4	26 Jan. 2025	WCO Bhutan	Dr Bhupinder Kaur Aulakh	WHO Representative
5	27 Jan. 2025	WCO Bhutan	Mr Ugyen Wangchuk	NPO (Admin./Planning/Prog. Mgt.)
6	27 Jan. 2025	UNDP	Khurshid Alam	Deputy Resident Representative
7	27 Jan. 2025	UNFPA	Jigme Choden	Gender focal officer
8	27 Jan. 2025	Department of Public Health Services, MoH	Mr Laigden Dzed	Chief Programme Officer
9	27 Jan. 2025	Department of Health Services	Mr Ugyen Tashi	Chief Programme Officer
10	27 Jan. 2025	UNICEF	Dr Indrani Chakma	Health and Nutrition Specialist
11	28 Jan. 2025	WCO Bhutan	Dr Lobzang Dorji	NPO (Family Health)
12	28 Jan. 2025	WCO Bhutan	Mr Pema Lethro	NPO (NCD)
13	28 Jan. 2025	WCO Bhutan	Mr Kencho Wangdi,	NPO (Emergency Programme)
14	28 Jan. 2025	WCO Bhutan	Ms Sonam Yangchen	Health Systems Focal Officer
15	28 Jan. 2025	National Medical Services	Dr Mimi Lhamu Mynak	President
16	28 Jan. 2025	Khesar Gyalpo University of Medical Sciences of Bhutan	Dr Dhakal	OIC
17	28 Jan. 2025	PEMA Secretariat	Dasho Dechen Wangmo	Head
18	28 Jan. 2025	Lhaksam	Mr Wangda	Executive Director
19	28 Jan. 2025	Disabled Peoples Organization	Mr Kinley Wangchuk	Executive Director
20	28 Jan. 2025	Bhutan Cancer Society	Mr Karma Togbay	Executive Director
21	28 Jan. 2025	Asian Development Bank	Mr Shokhimardon Musaev	Country Operations Head
22	28 Jan. 2025	Asian Development Bank	Ms Rada Dukpa	Social Development Officer
23	28 Jan. 2025	JICA	Ms Thuzi Jam	Project Officer
24	28 Jan. 2025	JICA	Mr Yohei Nuruki	Representative
25	28 Jan. 2025	JICA	Mr Tomoko Manabe	Project Formulation Adviser
26	28 Jan. 2025	Policy and Planning Division, MoH	Mr Tandin Dendup	Deputy Chief Planning Officer
27	28 Jan. 2025	HRD, MoH	Ms Ugyen Dema	HR Officer
28	29 Jan. 2025	SNV	Mr Kencho Wangdi	Portfolio Coordinator

	Date	Organization	Name	Position
29	29 Jan. 2025	Save the Children	Dr Tashi and Ms Yangchen Lodey Thinley	Head of Operations, and Policy and Social Protection
30	29 Jan. 2025	Ministry of Health	Dasho Pemba Wangchuk	Secretary
31	29 Jan. 2025	Ministry of Finance	Mr Phuntsho Wangchuk	WHO Focal
32	29 Jan. 2025	Ministry of Finance	Ms Tshering Choden	WHO Focal (former)
33	31 Jan. 2025	Dept of Public Health MOH	Mr Karma Jamtsho	Director
34	31 Jan. 2025	Dept. of Health Services, MoH	Mr Kinga Jampel	Director
35	31 Jan. 2025	FAO	Mr Ken Shimizu	Acting Resident Coordinator
36	31 Jan. 2025	Office of the UN Resident Coordinator	Mr Timothy Wilson	Head of Office
37	17 Feb. 2025	WHO-SEARO	Dr Anuruddhe Thushara Ranasinghe	WHO-SEARO Planning Division

8.6 CCS 2026–2030 indicators and targets

Indicators	Baseline	Target	Unit	Means of Verification	Alignment
Mental health index	20.1	58.6	Number	The PEMA/Annual Health Bulletin/ Program Report	SDG, Health Sector 13 FYP
Mental health services coverage index	37.7	78.7	Number	The PEMA/Annual Health Bulletin/Program Report	SDG, Health Sector 13 FYP
Health status index	86.8	92.2	Number	NHS and Annual Health Bulletin	SDG, Health Sector 13 FYP
NCD risk factors index	55.1	>59.7	Number	STEPS survey, NHS	SDG, Health Sector 13 FYP
Number of targeted priority diseases of public health concern with sustained elimination and eradication status	5	Sustain	Number	Program data, MoH	SDG, Health Sector 13 FYP
Number of targeted priority diseases of public health concern including NTDs controlled and eliminated	0	12		Program data, MoH	SDG, Health Sector 13 FYP
Population using safely managed drinking-water services	70	90		STEPS survey, NHS	SDG, Health Sector 13 FYP
Percentage of population using safely managed sanitation service	81.3	100	Percent	STEPS survey, NHS	SDG, Health Sector 13 FYP
Universal health service coverage index	75	90	Percent	STEPS survey, NHS	SDG, Health Sector 13 FYP

Indicators	Baseline	Target	Unit	Means of Verification	Alignment
Percentage of health facilities fulfilling human resources as per the HR Standard	80	90	percent	National Health Assessment/Account	SDG, Health Sector 13 FYP
International Health Regulation (IHR) Core Capacity Index	63	>67	percent	National Health Assessment/Account	SDG, Health Sector 13 FYP
Total health expenditure as a percentage of GDP	2.8	4	percent	National Health Assessment/Account	SDG, Health Sector 13 FYP
Health information surveillance and population health needs Index	41	72.8	percent	Program report, HERS, PPD, MOH	SDG, Health Sector 13 FYP
Percentage of girls vaccinated before age 15 years and received full dose HPV vaccine	90	Sustain	Percent	Program report, MoH	13 FYP

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